EMS CLINICAL CHECKLISTS — BATCH 2

Paramedic level with EMT delegation · 2010 Bradycardia · 2011 Tachycardia · 3002 Major Trauma · 4002 Allergic Reaction / Anaphylaxis · 4017 Stroke

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

EDITION CHECK REQUIRED BEFORE ANY USE. The source file's cover says 'Last Update 07/01/2026'. Appendix A (PDF p.321) shows the 2026-2028 changes as Effective Nov. 1, 2026. 1001.1.h: after release of a new version, all previous versions are invalid. Confirm which edition is in force with agency clinical leadership (G-1).

SOURCE

Greater Miami Valley EMS Council standing orders, Regional Physician Advisory Board Region 3, file 2026protocol.pdf (328 pages). Page numbers = PDF viewer page index (1-328). Each protocol also prints its own 'Page x of y' footer; the printed footer is NOT used. Section numbers (e.g., 2008.2) are cited with every item.

CONTENTS OF THIS BATCH

Protocol	Title	Effective	Last modified	PDF pages used	Checkboxes	Not-specified lines
2010	Bradycardia	June 1, 2021	Feb. 11, 2024	PDF p.63 (+ monographs 8005 p.233, 8006/8007 pp.235-237, 8019 p.261, 8024 pp.271-272, 8031 p.285, 8042 p.307)	33	6
2011	Tachycardia	June 1, 2021	May 16, 2026	PDF pp.65-66 (+ monographs 8001 p.225, 8003 p.229, 8024 pp.271-272, 8026 p.275, 8029 p.281, 8031 p.285)	37	5
3002	Major Trauma	June 1, 2021	May 16, 2026	PDF p.71 (+ 1002 p.15, 1005 p.21)	22	5
4002	Allergic Reaction / Anaphylaxis	June 1, 2021	May 16, 2026	PDF pp.119-120 (+ monographs 8002 p.227, 8013 p.249, 8017 p.257, 8018 p.259, 8019 p.261, 8023 p.269, 8026 p.275, 8030 p.283; Appendix A p.322)	35	4
4017	Stroke	June 1, 2021	Jan. 21, 2024	PDF pp.151-152 (+ 1005 p.22)	27	4

HOW TO READ THE TAGS

Tag	Meaning (source)
MCP ORDER	Source marks the item with a diamond. Per 1001.5.d.iv, diamond items ALWAYS require a physician's order (PDF p.12).
{ } OPTION	Source brackets the item. Per 1001.5.d.v, bracketed items are at the option of the agency and its Medical Director (PDF p.12).
EMR+ / EMT+ / AEMT+ / PM	Lowest level at which the source's colored level table lists the item. Levels are cumulative: the table lists directives 'for the EMT and above', etc. (1001.6, PDF p.13).
A / P / G	Source markers: A = adult only, P = pediatric (pink), G = geriatric (1001.5.d.vi-viii, PDF p.12).

WHAT THE DIAMOND AND BRACE MARKS DO NOT TELL YOU

Your department's Medical Director decides which { } options are adopted (1001.4.d: nothing in the protocol may be used without specific pre-approval of the local Medical Director). 1001.4.e-f: the protocol is used as clinically indicated; not every standing order applies to every patient, and discretionary judgment is required.

STRUCTURE OF THIS FILE

Cover; five checklists in the order above (each starts with its own title bar and status banner and follows the seven-heading template); REQUIRES HUMAN VERIFICATION (full text of every gap and conflict); SOURCE-VERIFICATION TABLES (one row per checklist line). Use the companion HTML file for collapsible, tappable checklists.

AUDIT SUMMARY

Automated source audit at build: 167 checklist items in this file - every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures; whole-document audit 335 items, 0 failures); 181 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 24 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

Contrast check of the print palette (WCAG ratio): navy on white 11.48:1; body on white 17.06:1; teal on white 4.99:1; red on white 5.44:1; cream on navy 10.42:1; white on navy 11.48:1.

CARDIAC PROTOCOL 2010 — BRADYCARDIA

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Bradycardia	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Cardiac Protocol 2010 — Bradycardia	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Feb. 11, 2024
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.63 (+ monographs 8005 p.233, 8006/8007 pp.235-237, 8019 p.261, 8024 pp.271-272, 8031 p.285, 8042 p.307)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

DEFINITION AND PRESENTATION (2010.1, 2010.2)

☐	**ALL**	Bradycardia is any heart rate less than 60 bpm.	2010.1.a · p.63
☐	**ALL**	Non-symptomatic bradycardia may be a normal finding in otherwise healthy individuals. Treat UNEXPLAINED or SYMPTOMATIC bradycardia.	2010.1.b, 2010.1.d · p.63
☐	**EMR+**	Signs & symptoms: heart rate less than 60 bpm; syncope; unstable bradycardia.	2010.2 Signs & Symptoms · p.63
☐	**EMR+**	Unstable bradycardia is listed with: hypotension; altered mental status; unresolved chest pain; poor skin color; diaphoresis.	2010.2 Signs & Symptoms · p.63
	ALL	Listed differential: acute myocardial infarction; hypoxia; hypothermia; elevated ICP (stroke or trauma); spinal cord lesion; sick sinus syndrome; athletic patients.	2010.2 Differential Diagnosis · p.63
☐	**ALL**	PEDIATRIC: with adequate perfusion, monitor vital signs and apply oxygen if needed. Hypoxia in pediatric patients will produce bradycardia.	2010.2 Pediatric Considerations · p.63

2 ASSESSMENT

☐	**ALL**	Assess the patient and determine medical history.	2010.1.c · p.63
☐	**EMT+** **{ } OPTION**	Obtain {12 Lead EKG}.	2010.2 Tx Algorithm, EMT block · p.63
☐	**PM** **{ } OPTION**	Obtain and interpret {12/15 Lead EKG}. *Condition / note: EKG wording differs by level block (see U-2010-9).*	2010.2 Tx Algorithm, Paramedic block · p.63
☐	**EMT+**	PEDIATRIC less than 2 years old: look for signs and symptoms of shock or hypoperfusion.	2010.2 Tx Algorithm, EMT block · p.63

Required assessment elements and vital-sign frequency for bradycardia (beyond 'observe and monitor vital signs'): NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	Administer oxygen as indicated.	2010.2 Tx Algorithm, EMR block · p.63

EMT LEVEL AND ABOVE

☐	**EMT+**	PEDIATRIC less than 2 years old: secure the airway and ventilate with BVM at 1 breath every 3-4 seconds.	2010.2 Tx Algorithm, EMT block · p.63

AEMT LEVEL AND ABOVE

	AEMT+	No additional orders at this level (source).	2010.2 Tx Algorithm, AEMT block · p.63

PARAMEDIC: HYPERKALEMIA CONSIDERATION

☐	**PM**	Wide complex bradycardia patients should spark consideration of treatment of hyperkalemia.	2010.2 Tx Algorithm, Paramedic block · p.63
☐	**PM** **MCP ORDER**	ADULT, MCP ORDER (paramedic consults): administer BOTH Calcium Chloride 10% 1 g (Calcium Chloride or Gluconate) AND Sodium Bicarbonate 100 mEq. *Condition / note: Source states no dialysis requirement; 8006 lists bradycardia only for renal dialysis patients (see U-2010-1).*	2010.2 Tx Algorithm, Paramedic block; Consult · p.63 8006 Adult Dosing · p.235 8042 Adult Dosing · p.307
☐	**PM**	Flush well between these medications. It is critical that these drugs not be given together, as they will precipitate.	2010.2 Tx Algorithm, Paramedic block · p.63 8006 Precautions · p.235

Pediatric calcium / sodium bicarbonate dosing for bradycardia (2010 gives adult dosing only): NOT SPECIFIED IN SOURCE — VERIFY.

PARAMEDIC: POOR PERFUSION IN ADULTS AND PEDIATRICS

☐	PM	ADULT: consider Atropine 1 mg IV, up to a total of 3 mg. *Condition / note: Interval between adult doses is not stated in 2010 or 8005 (see U-2010-2). Source lists this under 'poor perfusion in adults and pediatrics' with an adult marker.*	2010.2 Tx Algorithm, Paramedic block · p.63 8005 Adult Dosing · p.233
		Repeat interval between adult atropine doses: NOT SPECIFIED IN SOURCE — VERIFY.	
☐	PM	If treatments are ineffective, begin pacing.	2010.2 Tx Algorithm, Paramedic block · p.63
☐	PM	ADULT pacing, if time permits: Ketamine 25 mg IV (preferred method) OR Midazolam 2.5 mg slow IV prior to pacing. *Condition / note: 8024 lists suspected cardiac chest pain and acute MI as ketamine contraindications (see U-2010-6).*	2010.2 Tx Algorithm, Paramedic block · p.63 8024 Adult Dosing · p.271 8031 Adult Dosing · p.285
☐	PM	DO NOT reduce Ketamine or Midazolam doses by half for patients greater than 69 y/o.	2010.2 Tx Algorithm, Paramedic block · p.63 8024 Adult Dosing · p.271
☐	PM	ADULT pacing settings: set at 70 BPM, 20 mA and increase until mechanical capture is obtained. *Condition / note: No maximum current is stated for adults (see U-2010-7).*	2010.2 Tx Algorithm, Paramedic block · p.63
☐	PM	PEDIATRIC: Epinephrine (1:10,000) 0.01 mg/kg IV, repeat every 5 minutes. *Condition / note: 8019: repeat every 3-5 minutes, max single dose 1 mg (see U-2010-4).*	2010.2 Tx Algorithm, Paramedic block · p.63 8019 Pediatric Dosing · p.261
☐	PM	PEDIATRIC, if AV block: consider Atropine 0.02 mg/kg IV (minimum dose 0.1 mg, maximum single dose 0.5 mg). May repeat every 5 minutes. Max total dose 1 mg. *Condition / note: 8005 precaution: paradoxical bradycardia at doses less than 0.5 mg (see U-2010-3).*	2010.2 Tx Algorithm, Paramedic block · p.63 8005 Pediatric Dosing · p.233
☐	PM	PEDIATRIC, if AV block, consider pacing: pediatric electrodes on patients less than 15 kg; consider Midazolam 0.1 mg/kg (max dose 2 mg) slow IV prior to pacing; start with 5 mA, increasing as needed to 200 mA, at a rate of 80 bpm until capture. *Condition / note: 8031 gives a 2.5 mg maximum for this use (see U-2010-5).*	2010.2 Tx Algorithm, Paramedic block · p.63 8031 Pediatric Dosing · p.285

MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 2010 ITSELF)

☐	PM	ATROPINE (8005) contraindications: tachycardia; hypersensitivity to atropine; obstructive disease of GI tract; obstructive neuropathy; unstable cardiovascular status in acute hemorrhage with myocardial ischemia; narrow angle glaucoma; thyrotoxicosis. Precaution: paradoxical bradycardia when pushed too slowly or at doses less than 0.5 mg.	8005 Contraindications / Precautions · p.233
☐	PM	CALCIUM (8006): contraindications none in the emergency setting; do not administer with Sodium Bicarbonate (precipitate); flush tubing between drugs. Side effects include bradycardia (may cause asystole), hypotension, and severe local necrosis and sloughing following IV infiltration.	8006 Contraindications / Precautions · p.235
☐	PM	SODIUM BICARBONATE (8042) precautions: metabolic alkalosis; hypoxia; rise in intracellular PCO2 and increased tissue acidosis; hypernatremia; seizures; tissue sloughing at injection site. Contraindications: none in the emergency setting.	8042 Contraindications / Precautions · p.307
☐	PM	KETAMINE (8024) contraindications: suspected cardiac chest pain; hypertensive crisis; when significant elevations in BP might prove harmful (acute myocardial infarction, angina pectoris, aortic dissection). Provide continuous cardiac monitoring, EtCO2 and pulse oximetry with sedated patients. *Condition / note: See U-2010-6.*	8024 Contraindications / Precautions · p.272
☐	PM	MIDAZOLAM (8031): contraindication respiratory distress; can cause respiratory depression; monitor respirations and ventilate if necessary; provide continuous cardiac monitoring, EtCO2 and pulse oximetry with sedated patients.	8031 Contraindications / Precautions · p.285

EMT DELEGATION — from the source level tables

MAY	EMT: administer oxygen as indicated; call for transport immediately; observe and monitor vital signs if perfusion is adequate; {obtain the 12 Lead EKG}, transmit and call the receiving facility; transport immediately unless ALS intercept is less than 5 minutes (2010.2 EMR / EMT blocks, PDF p.63).
MAY	EMT, PEDIATRIC less than 2 years old: look for signs and symptoms of shock or hypoperfusion; secure the airway and ventilate with BVM at 1 breath every 3-4 seconds; if heart rate and perfusion do not increase within 30 to 60 seconds, perform CPR (2010.2 EMT block).
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Interpretation of the {12/15 Lead EKG}; calcium chloride / gluconate and sodium bicarbonate; atropine; epinephrine; transcutaneous pacing and pre-pacing sedation (all Paramedic block, 2010.2).

4 REASSESSMENT / RESPONSE

☐	EMR+	For adequate perfusion: observe and monitor vital signs.	2010.2 Tx Algorithm, EMR block · p.63

☐	**EMT+**	PEDIATRIC less than 2 years old: if heart rate and perfusion do not increase within 30 to 60 seconds, then perform CPR. *Condition / note: No heart-rate threshold is stated for starting CPR (see U-2010-8).*	2010.2 Tx Algorithm, EMT block · p.63

Reassessment interval, response-to-treatment criteria and decision points for repeating or escalating therapy: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMR+**	Call for transport immediately.	2010.2 Tx Algorithm, EMR block · p.63
☐	**EMT+**	Transport immediately unless ALS intercept is less than 5 minutes.	2010.2 Tx Algorithm, EMT block · p.63
☐	**EMT+** **{ } OPTION**	Transmit the {12 Lead EKG} and call the receiving facility.	2010.2 Tx Algorithm, EMT block · p.63
☐	**PM** **MCP ORDER**	Paramedic consults MCP for administration of Calcium Chloride 10% (or Gluconate) and Sodium Bicarbonate.	2010.2 Consult · p.63

Destination / receiving-facility selection for bradycardia and required handoff content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for bradycardia: NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-2010-1	2010.2 Paramedic vs 8006 / 8042 (p.235)	2010 directs Calcium Chloride 10% 1 g AND Sodium Bicarbonate 100 mEq for 'wide complex bradycardia' with...
U-2010-2	2010.2 / 8005 (p.233)	Adult atropine: 'Consider Atropine 1 mg IV, up to total of 3 mg' — the interval between doses is not stated in 2010...
U-2010-3	2010.2 vs 8005 (p.233)	The adult atropine line sits under 'poor perfusion in adults and pediatrics' with an adult marker; pediatric...
U-2010-4	2010.2 vs 8019 (p.261)	Pediatric epinephrine: 2010 says 'repeat every 5 minutes'; 8019 says 'repeat every 3-5 minutes (max single dose 1 mg)'
U-2010-5	2010.2 vs 8031 (p.285)	Pediatric pre-pacing midazolam: 2010 says 'max dose 2 mg' slow IV; 8031 says 0.1 mg/kg slow IV, 'maximum dose 2.5 mg'
U-2010-6	2010.2 vs 8024 (p.272)	2010 names Ketamine 25 mg IV as the PREFERRED pre-pacing agent in adults and lists acute myocardial infarction in...
U-2010-7	2010.2 Paramedic (p.63)	Adult pacing: start 70 BPM and 20 mA 'increase until mechanical capture is obtained' — no maximum current, no method...
U-2010-8	2010.2 EMT block (p.63)	Pediatric less than 2 years old: 'perform CPR' if heart rate and perfusion do not increase within 30 to 60 seconds...
U-2010-9	2010.2 EMT vs Paramedic block (p.63)	{12 Lead EKG} (EMT block) vs {12/15 Lead EKG} (Paramedic block); both are bracketed agency options
U-2010-10	2010.2 Pearls (p.63)	Clinical Pearls: 'None'. Monograph contraindications and cautions are reproduced under Treatment for the Paramedic;...

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

CARDIAC PROTOCOL 2011 — TACHYCARDIA

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Tachycardia	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Cardiac Protocol 2011 — Tachycardia	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 16, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.65-66 (+ monographs 8001 p.225, 8003 p.229, 8024 pp.271-272, 8026 p.275, 8029 p.281, 8031 p.285)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

DEFINITION AND PRESENTATION (2011.1, 2011.2)

☐	**ALL**	Tachycardia is any heart rate greater than 100 bpm. Treat UNEXPLAINED or SYMPTOMATIC tachycardia.	2011.1.a, 2011.1.c · p.65
☐	**EMR+**	Signs & symptoms: heart rate greater than 100 bpm; dizziness; chest pain; shortness of breath; unstable tachycardia.	2011.2 Signs & Symptoms · p.65
☐	**EMR+**	Unstable tachycardia is listed with: hypotension; altered mental status thought to be due to tachycardic rhythms.	2011.2 Signs & Symptoms · p.65

☐	ALL	Hemodynamically unstable patients will typically present with: hypotension; altered mental status; diaphoresis or cool/clammy skin. *Condition / note: Differs from the Signs & Symptoms list (see U-2011-7).*	2011.2 Clinical Pearls · p.66
	ALL	Listed differential: myocardial infarction; electrolyte imbalance; exertion/pain/emotional stress; fever; hypoxia; hypovolemia or anemia; drug overdose; hyperthyroidism; pulmonary embolus.	2011.2 Differential Diagnosis · p.65
☐	ALL	PEDIATRIC: with adequate perfusion, monitor vital signs and apply oxygen if needed.	2011.2 Pediatric Considerations · p.65

2 ASSESSMENT

☐	ALL	Assess the patient and determine medical history.	2011.1.b · p.65
☐	EMT+ { } OPTION	Obtain {12 Lead ECG}.	2011.2 Tx Algorithm, EMT block · p.65
☐	PM { } OPTION	Obtain and interpret {12/15 Lead ECG}.	2011.2 Tx Algorithm, Paramedic block · p.65
☐	ALL	Rule out or treat compensatory and physiological causes prior to treating tachycardia in isolation.	2011.2 Clinical Pearls · p.66

Required assessment elements, vital-sign frequency and a rate or QRS-width definition for narrow vs wide complex and for 'rapid ventricular rate': NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	EMR+	Administer oxygen as indicated.	2011.2 Tx Algorithm, EMR block · p.65

AEMT LEVEL AND ABOVE

	AEMT+	No additional orders at this level (source).	2011.2 Tx Algorithm, AEMT block · p.65

PARAMEDIC: ADULT, STABLE

☐	PM	Stable, narrow complex, regular: vagal maneuvers.	2011.2 Tx Algorithm, Paramedic block · p.65
☐	PM	Stable, narrow complex, regular: Adenosine 6 mg rapid IVP, saline flush. *Condition / note: See U-2011-1 (flush volume, repeat count).*	2011.2 Tx Algorithm, Paramedic block · p.65 8001 Adult Dosing · p.225
☐	PM	If PSVT history and the patient advises it takes 12 mg of Adenosine, skip the 6 mg dose.	2011.2 Clinical Pearls · p.66 8001 Adult Dosing · p.225
☐	PM	Stable, atrial fibrillation with rapid ventricular rate (RVR): transport for further evaluation.	2011.2 Tx Algorithm, Paramedic block · p.65
☐	PM	Stable, wide complex, regular or irregular: Amiodarone 150 mg in 250 ml NS, IV over 10 minutes using 60 drop/ml tubing. *Condition / note: 8003 contraindications not stated in 2011 (see U-2011-4).*	2011.2 Tx Algorithm, Paramedic block · p.65 8003 Adult Dosing · p.229
☐	PM	If Amiodarone is not available, use Lidocaine 150 mg IV/IO.	2011.2 Tx Algorithm, Paramedic block · p.65 8026 Adult Dosing · p.275

PARAMEDIC: ADULT, UNSTABLE

☐	PM	Consider a sedative/analgesic prior to cardioversion: Ketamine 25 mg IV (preferred method) OR Midazolam 2.5 mg slow IV. *Condition / note: 8024 contraindications (see U-2011-5).*	2011.2 Tx Algorithm, Paramedic block · p.65 8024 Adult Dosing · p.271 8031 Adult Dosing · p.285
☐	PM	DO NOT reduce Ketamine or Midazolam doses by half for patients greater than 69 y/o.	2011.2 Tx Algorithm, Paramedic block · p.65
☐	PM	Cardioversion: 100, 200, 300, 360 J for monophasic or biphasic equivalent. *Condition / note: Synchronization is not stated on this line (see U-2011-3).*	2011.2 Tx Algorithm, Paramedic block · p.65
☐	PM MCP ORDER	MCP ORDER: if atrial fibrillation with RVR is unstable, cardioversion at 200 J for monophasic or biphasic equivalent.	2011.2 Tx Algorithm, Paramedic block · p.65 2011.2 Consult · p.66
☐	PM	Paramedics should NOT cardiovert: patients without hemodynamic changes; patients whose hemodynamic changes have other apparent causes.	2011.2 Clinical Pearls · p.66

PARAMEDIC: ADULT, POLYMORPHIC VENTRICULAR TACHYCARDIA (TORSADES DE POINTES)

☐	PM	If stable: Magnesium Sulfate 2 gm infused with macro-drip tubing over 10 minutes.	2011.2 Tx Algorithm, Paramedic block · p.65 8029 Adult Dosing · p.281
☐	PM	If unstable: UNSYNCHRONIZED cardioversion 100, 200, 300, 360 J for monophasic or biphasic equivalent.	2011.2 Tx Algorithm, Paramedic block · p.65

PARAMEDIC: PEDIATRIC

☐	PM	Stable pediatrics: vagal maneuvers (blowing through a straw or oxygen tubing, etc.).	2011.2 Tx Algorithm, Paramedic block · p.66

☐	**PM**	Unstable pediatrics: Adenosine 0.1 mg/kg rapid IVP (max dose 6 mg), saline flush.	2011.2 Tx Algorithm, Paramedic block · p.66 8001 Pediatric Dosing · p.225
☐	**PM**	If no response: Adenosine 0.2 mg/kg rapid IVP (max dose 12 mg), saline flush. Repeat x 1.	2011.2 Tx Algorithm, Paramedic block · p.66 8001 Pediatric Dosing · p.225
☐	**PM**	Consider cardioversion. If time permits: Midazolam 0.1 mg/kg slow IV (max dose 2.5 mg).	2011.2 Tx Algorithm, Paramedic block · p.66 8031 Pediatric Dosing · p.285
☐	**PM**	Pediatric cardioversion 1 J/kg. If no response, repeat cardioversion at 2 J/kg.	2011.2 Tx Algorithm, Paramedic block · p.66

Pediatric treatment for wide complex tachycardia, pediatric polymorphic VT / torsades, and pediatric atrial fibrillation with RVR: NOT SPECIFIED IN SOURCE — VERIFY.

MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 2011 ITSELF)

☐	**PM**	ADENOSINE (8001) contraindications: second or third degree AV block or sick sinus syndrome; hypersensitivity to adenosine. May produce bronchoconstriction in patients with asthma and bronchopulmonary disease. Transient sinus bradycardia, sinus pause or asystole are listed side effects.	8001 Contraindications / Precautions · p.225
☐	**PM**	AMIODARONE (8003) contraindications: pulmonary congestion; cardiogenic shock; hypotension (SBP less than 100); sensitivity to amiodarone. Continuous EKG monitoring is required. Pediatric: not indicated for stable wide complex tachycardia.	8003 Contraindications / Precautions · p.229
☐	**PM**	LIDOCAINE (8026) contraindications: hypersensitivity; second or third degree heart block in the absence of an artificial pacemaker. Use extreme caution in hepatic disease, heart failure, marked hypoxia, severe respiratory depression, hypovolemia or shock, incomplete heart block or bradycardia and atrial fibrillation.	8026 Contraindications / Precautions · p.275
☐	**PM**	MAGNESIUM SULFATE (8029) contraindications: hypersensitivity; any heart block except first degree. Administer with caution if SBP less than 90 mmHg. Continuous cardiac monitoring and frequent vital signs are required.	8029 Contraindications / Precautions · p.281
☐	**PM**	KETAMINE (8024) contraindications: suspected cardiac chest pain; hypertensive crisis; when significant elevations in BP might prove harmful (acute myocardial infarction, angina pectoris, aortic dissection). MIDAZOLAM (8031) contraindication: respiratory distress; provide continuous cardiac monitoring, EtCO2 and pulse oximetry with sedated patients. *Condition / note: See U-2011-5.*	8024 Contraindications · p.272 8031 Contraindications / Precautions · p.285

EMT DELEGATION — from the source level tables	
MAY	EMT: administer oxygen as indicated; call for transport immediately; {obtain the 12 Lead ECG}, transmit and call the receiving facility; transport immediately unless ALS intercept is less than 5 minutes (2011.2 EMR / EMT blocks, PDF p.65).
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Interpretation of the {12/15 Lead ECG}; vagal maneuvers; adenosine; amiodarone; lidocaine; magnesium sulfate; sedation; cardioversion (all Paramedic block, 2011.2).

4 REASSESSMENT / RESPONSE

Reassessment interval, response-to-treatment criteria (including after adenosine, drug therapy and each cardioversion) and post-conversion care: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMR+**	Call for transport immediately.	2011.2 Tx Algorithm, EMR block · p.65
☐	**EMT+**	Transport immediately unless ALS intercept is less than 5 minutes.	2011.2 Tx Algorithm, EMT block · p.65
☐	**EMT+** **{ } OPTION**	Transmit the {12 Lead ECG} and call the receiving facility.	2011.2 Tx Algorithm, EMT block · p.65
☐	**PM** **MCP ORDER**	Paramedics must consult MCP for orders to cardiovert unstable patients presenting atrial fibrillation with RVR.	2011.2 Consult · p.66

Destination / receiving-facility selection for tachycardia and required handoff content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for tachycardia: NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-2011-1	2011.2 vs 8001 (p.225)	Adult adenosine: 2011 says 'May repeat Adenosine 12 mg rapid IVP x 2, saline flush'
U-2011-2	2011.2 Paramedic block (p.65)	Stable narrow complex regular: vagal maneuvers and adenosine are listed as separate bullets; the sequence (try vagal...
U-2011-3	2011.2 Paramedic block (p.65)	Adult unstable line says 'Cardioversion: 100, 200, 300, 360 J'; the word 'unsynchronized' appears only for torsades
U-2011-4	2011.2 vs 8003 / 8001 (p.229)	2011 gives amiodarone for stable wide-complex tachycardia with no cautions
U-2011-5	2011.2 vs 8024 (p.272)	2011 says Ketamine 25 mg IV is the PREFERRED sedation before cardioversion; 8024 contraindications include suspected...
U-2011-6	2011.2 vs 8029 (p.281)	Torsades: 8029 indication is 'in adult patient (with or without a pulse)'; 2011 uses adult markers only and gives no...
U-2011-7	2011.2 S&S vs Pearls (p.66)	'Unstable' is described as hypotension or altered mental status (S&S list) and in Pearls as hypotension, altered...
U-2011-8	2011.2 Pediatric (p.66)	Pediatric adenosine: 'Repeat x 1' follows the 0.2 mg/kg dose (8001: 'May repeat x one'); whether the 0.1 mg/kg dose...
U-2011-9	2011.2 Appendix A (p.322)	Appendix A (PDF p.322) lists new 2011 language: transport stable AF with RVR; cardiovert unstable AF with RVR by MCP...
U-2011-10	2011.2 EMT block (p.65)	{12 Lead ECG} (EMT) vs {12/15 Lead ECG} (Paramedic); both are bracketed agency options

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

TRAUMA PROTOCOL 3002 — MAJOR TRAUMA

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Major Trauma	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Trauma Protocol 3002 — Major Trauma	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 16, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.71 (+ 1002 p.15, 1005 p.21)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION AND PRIORITY (3002.1)

☐	**EMR+**	Sign / symptom listed: significant injuries or life threats.	3002.1 Signs & Symptoms · p.71
☐	**EMR+**	Patients meeting criteria for transport to a Trauma Center are considered 'Load and Go.' *Condition / note: The Trauma Center criteria are not in 3002 (see U-3002-1).*	3002.1 Tx Algorithm, EMR block · p.71
☐	**EMR+**	Consider the trauma care mnemonic M.A.R.C.H. to establish treatment priorities.	3002.1 Tx Algorithm, EMR block · p.71
	ALL	Pediatric Considerations: None. Differential Diagnosis: None.	3002.1 Assessment · p.71

2 ASSESSMENT

☐	**EMR+**	M - Massive Hemorrhage: assess for and address significant bleeding.	3002.1 M.A.R.C.H. · p.71
☐	**EMR+**	A - Airway: assess for a patent airway.	3002.1 M.A.R.C.H. · p.71
☐	**EMR+**	R - Respirations: assess respiratory rate, quality and effort.	3002.1 M.A.R.C.H. · p.71
☐	**EMR+**	C - Circulation: perform a head-to-toe sweep for additional bleeding.	3002.1 M.A.R.C.H. · p.71

Required primary / secondary survey elements, vital signs, GCS procedure and the criteria that define a significant injury or life threat: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	A - Airway: ensure a patent airway using adjuncts and positioning.	3002.1 M.A.R.C.H. · p.71
☐	**EMR+**	R - Respirations: oxygenate the patient with 100% O2.	3002.1 M.A.R.C.H. · p.71
☐	**EMR+**	H - Hypothermia: prophylactically treat the patient for hypothermia.	3002.1 M.A.R.C.H. · p.71
☐	**EMR+**	Open pneumothorax: cover the wound with an occlusive dressing, tape down three sides.	3002.1 Other trauma care considerations · p.71
☐	**EMR+**	Tension pneumothorax: lift one side of any occlusive dressing.	3002.1 Other trauma care considerations · p.71
☐	**EMR+**	Flail chest: stabilize immediately with a gloved hand, then immobilize with a bulky dressing or towels taped to the chest.	3002.1 Tx Algorithm, EMR block · p.71

EMT LEVEL AND ABOVE

	EMT+	No additional orders at this level (source).	3002.1 Tx Algorithm, EMT block · p.71

AEMT LEVEL AND ABOVE (TENSION PNEUMOTHORAX)

☐	AEMT+	Use caution not to confuse right main stem intubation for a pneumothorax. *Condition / note: Level read from the green color bar (see U-3002-3).*	3002.1 Tx Algorithm, AEMT block · p.71
☐	AEMT+	Perform needle decompression as indicated: decompress the chest with a 14-gauge or larger angiocath (3 ¼ inch in the source). *Condition / note: Indications not defined (see U-3002-6).*	3002.1 Tx Algorithm, AEMT block · p.71
☐	AEMT+	Site options: fourth or fifth intercostal space in the mid-axillary line; OR second or third intercostal space in the mid-clavicular line (use nipple line as a guide).	3002.1 Tx Algorithm, AEMT block · p.71
☐	AEMT+	PEDIATRIC less than 8 years old: decompression site choice will ONLY be the second or third intercostal space at the mid-clavicular line.	3002.1 Tx Algorithm, AEMT block · p.71

PARAMEDIC

☐	PM { } OPTION	{With defined and specific indications, paramedics who have received appropriate training and Medical Director's approval may perform a finger thoracostomy in accordance to their specific protocols.} *Condition / note: Agency option; specific protocol is not in this document (see U-3002-5).*	3002.1 Tx Algorithm, Paramedic block · p.71
☐	ALL	For a pregnant patient in arrest, consider the need for manual uterine displacement.	3002.1 Clinical Pearls · p.71

Hemorrhage-control methods, IV / fluid orders, spinal motion restriction, pain management and other trauma interventions are NOT in 3002 itself — NOT SPECIFIED IN SOURCE — VERIFY (see U-3002-4).

EMT DELEGATION — from the source level tables

MAY	EMT: all EMR-level items in 3002.1 — M.A.R.C.H. priorities; 100% O2; hypothermia prevention; open pneumothorax occlusive dressing taped on three sides; lift one side of the occlusive dressing for tension pneumothorax; stabilize a flail chest with a gloved hand then a bulky dressing or towels (PDF p.71). 3002 EMT block: 'No additional orders at this level.'
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	Needle decompression (listed at AEMT level on the color bar); finger thoracostomy (Paramedic {option}).
NOTE	EMR / EMT and AEMT / Paramedic distinction: MCP contact with MIVT, ETA and GCS is a Consult item for all providers (3002.1).

4 REASSESSMENT / RESPONSE

☐	EMR+	C - Circulation: double check all bleeding interventions.	3002.1 M.A.R.C.H. · p.71

Reassessment interval, response criteria and worsening pathway: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	EMR+	Patients meeting criteria for transport to a Trauma Center are 'Load and Go.' *Condition / note: Criteria are not in 3002 (see U-3002-1).*	3002.1 Tx Algorithm, EMR block · p.71
☐	ALL	Consult: contact Medical Control and advise them of patient condition with MIVT, ETA, and GCS components. *Condition / note: MIVT is defined differently in 1002.3.b and 3001 (see U-3002-2).*	3002.1 Consult · p.71 1002.3.b · p.15

Destination / Trauma Center selection, notification system use and handoff content: NOT SPECIFIED IN SOURCE — VERIFY (3018, 3020 not reproduced).

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for major trauma: NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-3002-1	3002.1 Tx Algorithm (p.71)	'Patients meeting criteria for transport to a Trauma Center are considered Load and Go' — the criteria are in 3018...
U-3002-2	3002.1 Consult vs 1002.3.b vs 3001 (p.15)	MIVT is defined as 'Mechanism, Injuries, Vital Signs and Time' in 1002.3.b (PDF p.15) but as 'Mechanism of Injury,...
U-3002-3	3002.1 Tx Algorithm (p.71)	Level reading: needle decompression is printed in the GREEN (AEMT) block, the EMT block says 'No additional orders...
U-3002-4	3002.1 scope (p.71)	3002 contains only M.A.R.C.H. priorities, chest-injury considerations and a Consult line
U-3002-5	3002.1 Paramedic block (p.71)	Finger thoracostomy is a bracketed { } option 'in accordance to their specific protocols' — no protocol text,...
U-3002-6	3002.1 AEMT block (p.71)	Needle decompression: 'as indicated' — the signs of tension pneumothorax are not listed in 3002
U-3002-7	3002.1 Tx Algorithm (p.71)	Oxygen: 'oxygenate the patient with 100% O2' in 3002 vs SpO2-titrated oxygen (e.g., titrate to 94%) in other...

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

MEDICAL PROTOCOL 4002 — ALLERGIC REACTION / ANAPHYLAXIS

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Allergic Reaction / Anaphylaxis	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Medical Protocol 4002 — Allergic Reaction / Anaphylaxis	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified May 16, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.119-120 (+ monographs 8002 p.227, 8013 p.249, 8017 p.257, 8018 p.259, 8019 p.261, 8023 p.269, 8026 p.275, 8030 p.283; Appendix A p.322)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

GUIDELINES (4002.1)

☐	**ALL**	Epinephrine is the mainstay of anaphylaxis in allergic reaction treatment.	4002.1.a · p.119
☐	**ALL**	Epinephrine is particularly important in cases of any airway edema, hypotension, or when multiple body systems are involved.	4002.1.b · p.119
☐	**ALL**	Advanced age is not a contraindication to epinephrine.	4002.1.c · p.119

PRESENTATION (4002.2)

☐	**EMR+**	Signs & symptoms: itching; hoarseness or stridor; wheezing; respiratory distress; altered level of consciousness; cyanosis; pulmonary edema; facial/airway edema; urticaria/hives.	4002.2 Signs & Symptoms · p.119
	ALL	Listed differential: rash only; shock (vascular effect); angioedema; aspiration/airway obstruction; vasovagal event; asthma.	4002.2 Differential Diagnosis · p.119
☐	**ALL**	PEDIATRIC: epinephrine is dosed based on weight, not age.	4002.2 Pediatric Considerations · p.119
☐	**ALL**	PEDIATRIC: the protocol lists patients under 15 kg as pediatric, but patients equal to or greater than 30 kg get both the Adult EpiPen and the EpiPen Jr. or Epinephrine 0.5 mg, no matter what their age. *Condition / note: See U-4002-3.*	4002.2 Pediatric Considerations · p.119

2 ASSESSMENT

Required assessment steps, vital signs, history (allergen / exposure / prior epinephrine use) and severity criteria separating 'allergic reaction' from anaphylaxis: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	Provide O2 as needed.	4002.2 Tx Algorithm, EMR block · p.119
☐	**EMR+**	If allergic reaction and weight is equal to or greater than 30 kg: give BOTH Adult EpiPen AND EpiPen Jr. (adult and pediatric). *Condition / note: 8017: initial dose needs no order at any level (Medical Control: No).*	4002.2 Tx Algorithm, EMR block · p.119 8017 Adult / Pediatric Dosing · p.257
☐	**EMR+**	PEDIATRIC less than 15 kg: EpiPen Jr.	4002.2 Tx Algorithm, EMR block · p.119 8017 Pediatric Dosing · p.257

☐	**EMR+**	PEDIATRIC equal to or greater than 15 kg and less than 30 kg: Adult EpiPen.	4002.2 Tx Algorithm, EMR block · p.119 8017 Pediatric Dosing · p.257
☐	**EMR+**	If applicable, apply an ice pack.	4002.2 Tx Algorithm, EMR block · p.119
	EMT LEVEL AND ABOVE		
☐	**EMT+**	If the patient develops wheezing, assist with their prescribed metered dose inhaler OR Albuterol 2.5 mg and Ipratropium 0.5 mg, nebulized with O2 flowing at 8-10 LPM. *Condition / note: CONFLICT on whether the EMT needs an MCP order (see U-4002-1).*	4002.2 Tx Algorithm, EMT block · p.119 8002 Adult Dosing · p.227 8023 Adult Dosing · p.269
☐	**EMT+** **{ } OPTION**	{If allergic reaction and an absence of Epi-pens in the drug bag, EMTs are permitted to administer Epinephrine IM via a syringe. The EMT may only perform this skill after authorization and training from their Medical Director.}	4002.2 Tx Algorithm, EMT block · p.119 8018 Indications · p.259 4002.2 Clinical Pearls · p.120
☐	**EMT+** **{ } OPTION**	{Epinephrine (1:1,000) IM: equal to or greater than 30 kg 0.5 mg (adult and pediatric); equal to or greater than 15 kg and less than 30 kg 0.3 mg; less than 15 kg 0.15 mg.} *Condition / note: Source prints '0.5 IM' without the unit for the 30 kg line (see U-4002-4).*	4002.2 Tx Algorithm, EMT block · p.119 8018 Adult / Pediatric Dosing · p.259
	AEMT LEVEL AND ABOVE		
☐	**AEMT+**	If an allergic reaction: equal to or greater than 30 kg, give both Adult EpiPen and EpiPen Jr OR Epi (1:1,000) 0.5 mg IM (adult and pediatric).	4002.2 Tx Algorithm, AEMT block · p.119 8018 Adult Dosing · p.259
☐	**AEMT+**	PEDIATRIC equal to or greater than 15 kg and less than 30 kg: Adult EpiPen OR Epinephrine (1:1,000) 0.3 mg IM. Less than 15 kg: EpiPen Jr OR Epinephrine (1:1,000) 0.15 mg IM.	4002.2 Tx Algorithm, AEMT block · p.119 8018 Pediatric Dosing · p.259
☐	**AEMT+**	If apneic, intubate, possibly with smaller than normal ET tube. *Condition / note: 1008 Advanced Airway Management not reproduced (G-3).*	4002.2 Tx Algorithm, AEMT block · p.119
☐	**AEMT+**	If the patient is intubated, Albuterol 2.5 mg by nebulizer into the ETT. If Ipratropium was not given before intubation, add it to the first Albuterol.	4002.2 Tx Algorithm, AEMT block · p.119
☐	**AEMT+**	If hypotensive: ADULT IV fluid to maintain adequate BP. PEDIATRIC IV fluid 20 ml/kg IV to maintain adequate BP. *Condition / note: Adult volume and 'adequate BP' not defined (see U-4002-6).*	4002.2 Tx Algorithm, AEMT block · p.119
☐	**AEMT+**	ADULT: Diphenhydramine 50 mg IM or IV. PEDIATRIC: Diphenhydramine 1 mg/kg IM or IV (max dose 50 mg). *Condition / note: 8013 says 'IM or slow IV' (see U-4002-5).*	4002.2 Tx Algorithm, AEMT block · p.119 8013 Adult / Pediatric Dosing · p.249
	PARAMEDIC		
☐	**PM**	If the patient is deteriorating or unresponsive, consider early intubation, possibly with a smaller than normal size endotracheal tube.	4002.2 Tx Algorithm, Paramedic block · p.120
☐	**PM**	If a conscious patient requires intubation — ADULT: Lidocaine 100 mg IN, half dose per nostril, or added to nebulizer with breathing treatment. PEDIATRIC: Lidocaine 1.5 mg/kg nebulized with O2 8-10 LPM or IN; maximum dose 100 mg.	4002.2 Tx Algorithm, Paramedic block · p.120 8026 Adult / Pediatric Dosing · p.275
☐	**PM**	If the patient remains hypotensive after IV fluid: Epinephrine (1:10,000) 0.1 mg, slow IV, every 3 minutes up to 0.5 mg. *Condition / note: Not in the 8019 indications; no pediatric dose given (see U-4002-7).*	4002.2 Tx Algorithm, Paramedic block · p.120
☐	**PM**	Solu-Medrol: ADULT 125 mg IV. PEDIATRIC 2 mg/kg IV, max dose 125 mg.	4002.2 Tx Algorithm, Paramedic block · p.120 8030 Adult / Pediatric Dosing · p.283
☐	**PM**	Solu-Medrol is given to all patients treated within the allergic reaction or anaphylaxis protocol ONLY AFTER all other applicable first-line medications have been delivered. Do not initiate an IV only to administer this medication (8030).	4002.2 Clinical Pearls · p.120 8030 Precautions · p.283
	ALL	Pearl: no significant change in patient condition in the field should be expected from the administration of Solu-Medrol.	4002.2 Clinical Pearls · p.120
	MONOGRAPH CAUTIONS FOR DRUGS NAMED ABOVE (NOT STATED IN 4002 ITSELF)		
☐	**ALL**	EPINEPHRINE (8017, 8018, 8019): contraindications none in the emergency setting. Side effects include dysrhythmias (including VT and VF), hypertension, tachycardia, and increased myocardial oxygen demand or precipitation of angina pectoris.	8017 Contraindications / Precautions · p.257 8018 Contraindications / Precautions · p.259 8019 Contraindications / Precautions · p.261
☐	**AEMT+**	DIPHENHYDRAMINE (8013): contraindications none in the emergency setting; use cautiously in patients with CNS depression or lower respiratory diseases such as asthma; hypotension is a listed side effect.	8013 Contraindications / Precautions · p.249
☐	**EMT+**	ALBUTEROL (8002): prior hypersensitive reaction to albuterol; cardiac dysrhythmias associated with tachycardia. IPRATROPIUM (8023): contraindications none in the emergency setting; caution in narrow-angle glaucoma and lactating mothers.	8002 Contraindications · p.227 8023 Contraindications / Precautions · p.269
☐	**PM**	LIDOCAINE (8026): hypersensitivity; second or third degree heart block in absence of an artificial pacemaker; extreme caution with hepatic disease, heart failure, marked hypoxia, severe respiratory depression, hypovolemia or shock.	8026 Contraindications / Precautions · p.275

☐	**PM**	SOLU-MEDROL (8030): contraindications none in emergency setting; side effects cardiac arrhythmias and syncope.	8030 Contraindications / Precautions · p.283

EMT DELEGATION — from the source level tables	
MAY	EMT: provide O2 as needed; give EpiPen by weight (equal to or greater than 30 kg both Adult EpiPen and EpiPen Jr.; 15 to less than 30 kg Adult EpiPen; less than 15 kg EpiPen Jr.); apply an ice pack if applicable; call for transport; assist with the patient's prescribed metered dose inhaler (4002.2 EMR / EMT blocks, PDF p.119). 8017: initial EpiPen dose needs no order.
MCP ORDER	Repeat epinephrine (4002.2 Consult; 8017 Medical Control: repeat doses by EMR / EMTs — Yes).
MAY	EMT, {agency option}: if there are no Epi-pens in the drug bag, administer Epinephrine IM via syringe (weight-based 0.5 / 0.3 / 0.15 mg) ONLY after authorization and training from the Medical Director. Repeat is a diamond (MCP order) item.
NOTE	EMT nebulized Albuterol 2.5 mg + Ipratropium 0.5 mg at 8-10 LPM: the 4002 Consult line (PDF p.120) still says 'EMT needs MCP orders to administer breathing treatments', but Appendix A (PDF p.322) and monographs 8002 / 8023 say the EMT requirement was removed. CONFLICT — VERIFY before relying on either reading (U-4002-1).
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOT LISTED AT EMT LEVEL	IV fluid for hypotension; diphenhydramine; intubation and nebulized albuterol into the ETT; intranasal / nebulized lidocaine; IV epinephrine (1:10,000); Solu-Medrol.

4 REASSESSMENT / RESPONSE

☐	**EMR+ MCP ORDER (EMT)**	If symptoms persist, may repeat Epinephrine (adult and pediatric) in 10 minutes. MCP ORDER for EMR and EMT.	4002.2 Tx Algorithm, EMR block · p.119 4002.2 Consult · p.120 8017 Medical Control · p.257
☐	**EMT+ MCP ORDER (EMT) { } OPTION**	{EMT, syringe option: may repeat Epinephrine (1:1,000) 0.5 mg IM after 10 minutes (adult and pediatric); pediatric at weight appropriate dose after 10 minutes.} MCP ORDER.	4002.2 Tx Algorithm, EMT block · p.119 4002.2 Consult · p.120
☐	**EMT+**	EMT: may repeat Albuterol 2.5 mg nebulized two times. *Condition / note: Order requirement conflict (see U-4002-1).*	4002.2 Tx Algorithm, EMT block · p.119 8002 Adult Dosing · p.227
☐	**AEMT+**	AEMT / Paramedic: may repeat Epinephrine (1:1,000) 0.5 mg IM after 10 minutes (adult and pediatric); pediatric at weight appropriate dose after 10 minutes. No MCP order is printed for these levels. *Condition / note: Maximum number of doses not stated (see U-4002-2).*	4002.2 Tx Algorithm, AEMT block · p.119 8017 Medical Control · p.257

Reassessment interval, response-to-treatment criteria, biphasic reaction / observation, and maximum cumulative epinephrine doses: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMR+**	Call for transport.	4002.2 Tx Algorithm, EMR block · p.119

Destination, notification, MCP contact requirements other than repeat epinephrine, and handoff content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for allergic reaction / anaphylaxis (e.g., weight-based dose documentation, times of each epinephrine dose): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-4002-1	4002.2 Consult vs Appendix A vs 8002 / 8023 (p.120)	4002 Consult (p.120): 'EMT needs MCP orders to administer breathing treatments.' Appendix A (p.322): '4002.2 EMT —...
U-4002-2	4002.2 (p.119)	Repeat epinephrine: 'may repeat ... in 10 minutes' — the number of repeats and maximum total dose are not stated in...
U-4002-3	4002.2 Pediatric Considerations vs 8017 / 1005.4 (p.257)	The protocol 'lists those patients under 15 kg as pediatric' and dosing is by weight; 1005.4 defines pediatric as...
U-4002-4	4002.2 EMT block (p.119)	The EMT syringe line prints 'Epinephrine (1:1,000) 0.5 IM' without a unit (0.5 mg appears in the repeat line)
U-4002-5	4002.2 AEMT block vs 8013 (p.249)	Diphenhydramine: 4002 says 'IM or IV'; 8013 says 'IM or slow IV' (1005.4 defines slow IV as over 2 minutes unless...
U-4002-6	4002.2 AEMT block (p.119)	Hypotension: adult 'IV fluid to maintain adequate BP' has no volume, fluid type or BP definition; pediatric is 20...
U-4002-7	4002.2 Paramedic block vs 8019 (p.261)	Epinephrine (1:10,000) 0.1 mg slow IV every 3 minutes up to 0.5 mg appears with no adult / pediatric marker, no...
U-4002-8	4002.1 / 4002.2 (p.119)	The Treatment Algorithm starts 'If allergic reaction:' and gives epinephrine; 4002.1.a-b describe epinephrine as the...
U-4002-9	4002.2 AEMT / Paramedic blocks (p.119)	Intubation: 1008 Advanced Airway Management is not reproduced; 8031 says the AEMT should intubate if apneic
U-4002-10	Appendix A (p.322)	4002 'Last modified' is May 16, 2026; the cover says the book was last updated 07/01/2026 and Appendix A is...

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

MEDICAL PROTOCOL 4017 — STROKE

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Stroke	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Medical Protocol 4017 — Stroke	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Jan. 21, 2024
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.151-152 (+ 1005 p.22)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

STROKE ALERT CRITERIA (4017.1)

☐	**ALL**	If one or more signs of the Cincinnati Prehospital Stroke Scale (CPSS) are abnormal AND less than 24 hours since the patient was last seen normal: call a 'Stroke Alert' and transport to the closest appropriate Stroke Center. *Condition / note: 'Closest appropriate' is not defined (see U-4017-5).*	4017.1.a · p.151
☐	**ALL**	In addition to the CPSS, screen patients for possible large vessel occlusions (LVO) before making transport destination decisions.	4017.1.b · p.151
☐	**ALL**	If greater than 24 hours since last known well, consider transport to a Comprehensive or Thrombectomy Capable facility.	4017.1.c · p.151
☐	**ALL**	When reporting last known well, state the actual clock time. Do not say '20 minutes ago.'	4017.1.d · p.151
☐	**EMR+**	Signs & symptoms: facial drooping; arm drift or weakness; slurred or difficult speech; aphasia (expressive or receptive); pupillary changes (in hemorrhagic strokes); gaze deviation / abnormal eye movement (indicative of large vessel occlusions).	4017.2 Signs & Symptoms · p.151
	ALL	Listed differential: seizure; subdural hematoma; brain tumor; syncope; toxic or metabolic disorders (e.g., hypoglycemia); migraine headaches. Pediatric Considerations: None.	4017.2 Differential / Pediatric · p.151
	ALL	Agencies may, with Medical Director approval, use alternative screening scales (RACE, MEND, LAPSS, etc.) that include the routine CPSS assessments and a method to screen for LVO; may base destination on proximity and hospital capabilities; all changes must be a supplemental guideline signed off by the medical director.	4017.1.e · p.151

2 ASSESSMENT

☐	**EMR+** **{ } OPTION**	Perform a Cincinnati Pre-hospital Stroke Scale {or alternative approved by Medical Direction} assessment.	4017.2 Tx Algorithm, EMR block · p.151

☐	**EMR+**	CPSS (normal or abnormal): facial droop (patient shows teeth or smiles); arm drift (patient closes eyes and holds both arms straight out for about 10 seconds); abnormal speech (have patient say 'You can't teach an old dog new tricks.' or any other phrase).	4017.2 Clinical Pearls · p.152
☐	**EMT+**	Perform a Large Vessel Occlusion (LVO) screening looking for: difficulty in balance or gait; eye deviation (eyes may only move to one side, or be forced to one side); visual disturbances (field of view cut, double vision, new onset blindness); aphasia (expressive or receptive); denial / neglect (can a patient feel you touch both of their arms and do they recognize their own hand?).	4017.2 Tx Algorithm, EMT block · p.152
	ALL	LVO indicators (Pearls): abnormal findings in all three CPSS categories increase the possibility of LVO; visual neglect, gaze deviation or abnormal eye movement are key clinical findings; new onset loss of balance or coordination may indicate a possible LVO stroke.	4017.2 Clinical Pearls · p.152
☐	**EMT+**	Consider the following contraindications to thrombolytics: neurosurgery, head trauma or stroke in the last 3 months; major surgery or serious non-head trauma in the previous 14 days; history of gastrointestinal or urinary tract hemorrhage within 21 days; current (within the last 48 hours) use of anticoagulants.	4017.2 Tx Algorithm, EMT block · p.152
☐	**EMT+**	Anticoagulant examples listed: Warfarin (Coumadin, Jantoven); Apixiban [sic] (Eliquis); Abigatran [sic] (Pradaxa); Edoxaban (Savaysa); Rivaroxaban (Xarelto); Lovenox injections. *Condition / note: Source spellings; see U-4017-7.*	4017.2 Tx Algorithm, EMT block · p.152
☐	**EMR+**	Perform blood glucose check (1005 General Patient Management). *Condition / note: 4017 itself only says 'If glucose is less than 60...' (see U-4017-6).*	1005.4 Tx Algorithm, EMR block · p.22

Time-of-onset / last-known-well interview questions, medication and anticoagulant history prompts, and vital-sign requirements beyond the items above: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

EMR LEVEL AND ABOVE

☐	**EMR+**	A patient in respiratory distress with pale, moist skin and altered mental status should get oxygen via NRB mask. *Condition / note: Interaction with the SpO2 rule is not stated (see U-4017-2).*	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+**	Be prepared to assist ventilations with OPA/NPA and bag-valve-mask.	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+**	If signs of cerebral herniation are present, ventilate ADULT at approximately 20 times per minute. *Condition / note: Signs of herniation are not defined (see U-4017-3).*	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+**	PEDIATRIC with signs of cerebral herniation: ventilate at a rate of ten faster than the normal respiratory rate. *Condition / note: Wording is ambiguous (see U-4017-3).*	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+** **{ } OPTION**	{If numeric EtCO2 readings are available, ventilate at a rate to maintain readings at approximately 30 mmHg (30 torr).}	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+**	Never ventilate at less than 8 per minute.	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+**	Patient with indications of stroke and SpO2 less than 94%: oxygen via NC and titrated to 94%. *Condition / note: Exactly 94% is not addressed (see U-4017-1).*	4017.2 Tx Algorithm, EMR block · p.151
☐	**EMR+**	Patient with indications of stroke and SpO2 greater than 94%: should not get any oxygen. *Condition / note: Exactly 94% is not addressed (see U-4017-1).*	4017.2 Tx Algorithm, EMR block · p.151

EMT LEVEL AND ABOVE

☐	**EMT+**	If glucose is less than 60, or there is strong suspicion of hypoglycemia despite glucometer readings, then follow the 4008 Diabetic Emergencies - Hypoglycemia protocol. *Condition / note: 4008 not reproduced (G-3).*	4017.2 Tx Algorithm, EMT block · p.152
☐	**EMT+**	Transport the patient with the bed flat, if able to tolerate. If showing signs of increased ICP, do not lay the patient flat.	4017.2 Tx Algorithm, EMT block · p.152

AEMT LEVEL AND PARAMEDIC

	AEMT+	No additional orders at this level (source prints this for AEMT and for Paramedic).	4017.2 Tx Algorithm, AEMT and Paramedic blocks · p.152

EMT DELEGATION — from the source level tables

MAY	EMT: every treatment-algorithm item in 4017.2 is at the EMR or EMT level — CPSS (or approved alternative), oxygen by SpO2 rule / NRB for distress with pale, moist skin and altered mental status, ventilation assist with OPA/NPA and BVM, LVO screen, thrombolytic-contraindication review, Stroke Alert and destination per 4017.1 / 4017.2, bed-flat transport unless signs of increased ICP, and 4008 if glucose is less than 60 (PDF pp.151-152). AEMT and Paramedic blocks: 'No additional orders at this level.'
MCP ORDER	EMT, MCP: contact MCP for Stroke Alerts or for advice regarding transport destination if not clear (4017.2 Consult).

MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
NOTE	No medication is given in 4017; AEMT and Paramedic have no additional orders. The EMT carries the full field role.

4 REASSESSMENT / RESPONSE

Reassessment interval, response criteria and worsening pathway (including seizure, airway or glucose changes): NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**EMT+**	The presence of a single abnormal finding in the CPSS {or alternative screening approved by Medical Direction} should dictate a stroke alert and transport to the closest stroke center (unless contraindicated by greater than 24-hour onset, presence of LVO indicators or thrombolytic considerations). *Condition / note: See U-4017-5.*	4017.2 Tx Algorithm, EMT block · p.152
☐	**EMT+**	Patients with onset greater than 24 hours, clinical findings indicative of LVO, or contraindications to thrombolytics: consider transport to a Comprehensive Stroke Center or Thrombectomy Capable Facility.	4017.2 Tx Algorithm, EMT block · p.152
☐	**EMT+**	Patients with signs or symptoms that strongly indicate a possible hemorrhagic stroke should NOT be transported to a Thrombectomy Capable Facility. *Condition / note: Overlap with 'Comprehensive' centers (see U-4017-4).*	4017.2 Tx Algorithm, EMT block · p.152
☐	**ALL**	Contact MCP for Stroke Alerts or for advice regarding transport destination, if not clear.	4017.2 Consult · p.152
☐	**ALL**	Arrange for a historian to be transported with the patient, both to provide patient history and for permission to treat. *Condition / note: Source wording is garbled (see U-4017-8).*	4017.2 Clinical Pearls · p.152
	ALL	Stroke Centers (4017.3): Telemedicine Stroke Center (drip and ship; thrombolytic capabilities; immediate neurologist access via telemedicine). Primary Stroke Center (can administer thrombolytics; has an ICU). Comprehensive or Thrombectomy Capable (24/7 endovascular capabilities): Miami Valley Hospital (Comprehensive); Kettering (Comprehensive); Mercy Health - Springfield (Thrombectomy Capable).	4017.3.a-c · p.152

Receiving-facility notification method / number for a Stroke Alert and required handoff content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

☐	**ALL**	Record the last known well as an actual clock time (not an elapsed interval).	4017.1.d · p.151

Other required documentation elements (CPSS results, LVO screen result, glucose, thrombolytic-contraindication findings, destination rationale): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-4017-1	4017.2 EMR block (p.151)	Oxygen: 'SpO2 less than 94% ... titrated to 94%' and 'SpO2 greater than 94% ..
U-4017-2	4017.2 EMR block (p.151)	NRB mask for 'respiratory distress with pale, moist skin and altered mental status' vs the SpO2 rule (no oxygen...
U-4017-3	4017.2 EMR block (p.151)	'Signs of cerebral herniation' are not defined in 4017
U-4017-4	4017.2 EMT block vs 4017.3.c (p.152)	'Comprehensive Stroke Centers or Thrombectomy Capable' are defined together (4017.3.c), and Comprehensive centers...
U-4017-5	4017.1.a / 4017.2 (p.151)	'Closest appropriate Stroke Center' is not defined; only the Comprehensive / Thrombectomy centers are named (4017.3.c)
U-4017-6	4017.2 EMT block (p.152)	Glucose: 4017 gives only a conditional ('If glucose is less than 60, or strong suspicion of hypoglycemia'); the...
U-4017-7	4017.2 EMT block (p.152)	Thrombolytic-contraindication list uses source spellings 'Apixiban' and 'Abigatran'; the visible page prints...
U-4017-8	4017.2 Pearls (p.152)	'Arrange for transport a historian with patient' is garbled; it is not clear whether the historian is to ride with...
U-4017-9	4017.2 / Appendix A (p.322)	4017 'Last modified' is Jan. 21, 2024 and does not appear in Appendix A's 2026-2028 changes

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

REQUIRES HUMAN VERIFICATION

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

Global issues (all protocols in this file)

ID	Topic	Issue	Clarification needed
G-1	Edition / effective date	Cover (PDF p.1): 'Last Update 07/01/2026'. Appendix A (PDF p.321): '2026-2028 Protocol Changes', Effective Nov. 1, 2026. Each protocol prints 'Effective: June 1, 2021' and its own 'Last modified'. 1001.1.h: after release of a new version, all previous versions are invalid. Today is Oct. 30, 2026.	Confirm with agency clinical leadership which edition is in force today and on Nov. 1, 2026 before any field use.
G-2	Level mapping method	Provider levels come from the colored side bars of each Clinical Management Table (grey EMR, orange EMT, green AEMT, blue Paramedic). Text extraction cannot carry that information, so levels were read from rendered page images and cross-checked against Appendix A and the monographs.	Reviewer should spot-check level tags against the printed color bars.
G-3	Cross-referenced protocols not reproduced	Referenced but NOT built or reviewed in Batches 1-2: 1005 (only quoted lines used), 1008/1009 (airway), 1012 (IO), 1014 (only quoted lines), 3001, 3015, 3017, 3018, 3020, 4003, 4005, 4008, 4011, 4016 (shock), 8035 (norepinephrine), 8036 (fluids), 7012/7013, 3019/3021. Batch 2 built 2010, 2011, 3002, 4002, 4017.	Tell me which to add in later batches.
G-4	Agency options and Medical Director approval	Items in { } are agency/Medical Director options (1001.5.d.v) and 1001.4.d says nothing may be used without Medical Director pre-approval. This checklist cannot know which options your department has adopted. 1001.4.e-f: protocol is used as clinically indicated; not every standing order applies to every patient.	Agency Medical Director to mark adopted options.
G-5	Monograph level columns	Drug monographs (8000 series) print level tabs as column headers; the text extraction does not show which column each row sits in. Medication level statements in the checklist rely on the protocol tables, with monographs used for doses, contraindications and Medical Control lines.	Reviewer to confirm any level-specific monograph statement.
G-6	Status	This checklist has NOT been reviewed or approved by agency clinical leadership and does not authorize care. Reviewer sign-off lines in Section 7 are intentionally blank.	Complete the review sign-off.
G-7	Monograph cross-check scope and numbering	Batch 2 doses were compared with monographs 8001, 8002, 8003, 8005, 8006/8007, 8013, 8017, 8018, 8019, 8023, 8024, 8026, 8029, 8030, 8031 and 8042 (Medical Control lines, dosing, contraindications). Footer / header numbers are inconsistent in the source: the Calcium Gluconate page header reads 8007 but its footer reads '8006 - Calcium Gluconate'; the Norepinephrine page header reads 8035 but its footer reads '8034 - Norepinephrine'; Nitroglycerin is 8034. Monograph level columns were not read (G-5).	Reviewer: confirm drug and number mapping; no clinical content was inferred from the numbering.

2010 Bradycardia

ID	Source location	Issue	Clarification needed
U-2010-1	2010.2 Paramedic vs 8006 / 8042 (p.235)	2010 directs Calcium Chloride 10% 1 g AND Sodium Bicarbonate 100 mEq for 'wide complex bradycardia' with hyperkalemia consideration and no dialysis requirement. 8006 lists bradycardia only for a RENAL DIALYSIS patient (diamond, Medical Control Yes). 8042 lists no bradycardia / hyperkalemia indication (its Indications are arrest in dialysis, excited delirium arrest, tricyclic overdose, crush syndrome). 'Calcium Chloride or Gluconate' is given a single dose (1 g); 8006 and 8007 both list 1 gm (10 ml).	Medical Director: confirm indication, whether dialysis status is required, and how hyperkalemia is to be suspected in the field.
U-2010-2	2010.2 / 8005 (p.233)	Adult atropine: 'Consider Atropine 1 mg IV, up to total of 3 mg' — the interval between doses is not stated in 2010 or in 8005 (pediatric interval is stated: every 5 minutes).	Confirm adult repeat interval.
U-2010-3	2010.2 vs 8005 (p.233)	The adult atropine line sits under 'poor perfusion in adults and pediatrics' with an adult marker; pediatric atropine appears only under 'If AV block'. 8005 pediatric bradycardia dosing has no AV-block condition. 8005 warns of paradoxical bradycardia at doses less than 0.5 mg while the pediatric minimum single dose in both documents is 0.1 mg.	Confirm whether pediatric atropine is limited to AV block and how the 0.1 mg minimum is to be used.
U-2010-4	2010.2 vs 8019 (p.261)	Pediatric epinephrine: 2010 says 'repeat every 5 minutes'; 8019 says 'repeat every 3-5 minutes (max single dose 1 mg)'. 8019 lists no adult bradycardia indication (2010 lists none either).	Confirm repeat interval.
U-2010-5	2010.2 vs 8031 (p.285)	Pediatric pre-pacing midazolam: 2010 says 'max dose 2 mg' slow IV; 8031 says 0.1 mg/kg slow IV, 'maximum dose 2.5 mg'. (2011 pediatric cardioversion sedation also says max 2.5 mg.)	Confirm correct maximum.
U-2010-6	2010.2 vs 8024 (p.272)	2010 names Ketamine 25 mg IV as the PREFERRED pre-pacing agent in adults and lists acute myocardial infarction in the differential. 8024 Contraindications list suspected cardiac chest pain, hypertensive crisis and acute MI / angina / aortic dissection ('when significant elevations in BP might prove harmful'). 1014.1.d (PDF p.41) also says ketamine is not to be given for suspected cardiac chest pain. Whether these apply to pacing sedation is not stated.	Medical Director: does the 'preferred' status override the contraindication list when pacing a patient with possible AMI?
U-2010-7	2010.2 Paramedic (p.63)	Adult pacing: start 70 BPM and 20 mA 'increase until mechanical capture is obtained' — no maximum current, no method to confirm mechanical capture and no rate adjustment are stated. Pediatric pacing gives a range (5 mA up to 200 mA, 80 bpm).	Medical Director clarification if needed.
U-2010-8	2010.2 EMT block (p.63)	Pediatric less than 2 years old: 'perform CPR' if heart rate and perfusion do not increase within 30 to 60 seconds of ventilation — no heart-rate threshold for starting CPR is given. Bradycardia is defined once as 'less than 60 bpm' with no age-specific values (2010.1.a).	Confirm threshold for CPR in this age group.
U-2010-9	2010.2 EMT vs Paramedic block (p.63)	{12 Lead EKG} (EMT block) vs {12/15 Lead EKG} (Paramedic block); both are bracketed agency options. 'Unstable bradycardia' (S&S list) and 'poor perfusion' / 'adequate perfusion' (Treatment Algorithm) are not defined relative to each other.	Agency: which 12-lead option is adopted? Medical Director: define the perfusion terms.
U-2010-10	2010.2 Pearls (p.63)	Clinical Pearls: 'None'. Monograph contraindications and cautions are reproduced under Treatment for the Paramedic; whether they apply to each use in bradycardia has not been confirmed. 4016 Shock, 1012 IO and 8036 fluids are not reproduced.	Reviewer: confirm applicability.

2011 Tachycardia

ID	Source location	Issue	Clarification needed
U-2011-1	2011.2 vs 8001 (p.225)	Adult adenosine: 2011 says 'May repeat Adenosine 12 mg rapid IVP x 2, saline flush'. 8001 says 6 mg, 'if not successful, may repeat 12 mg rapid IV' and, for a PSVT-history patient going directly to 12 mg, 'May repeat once'. 'x 2' could mean two additional 12 mg doses. Flush: 2011 says 'saline flush' (adult); 8001 says 'followed by 20 ml bolus of IV fluid' (adult) and '10 ml rapid saline flush' (pediatric). 8001 indication is 'Stable PSVT'; 2011 says 'Narrow Complex - Regular'.	Medical Director: confirm number of 12 mg doses and flush volume.
U-2011-2	2011.2 Paramedic block (p.65)	Stable narrow complex regular: vagal maneuvers and adenosine are listed as separate bullets; the sequence (try vagal maneuvers first?) and the duration or technique of vagal maneuvers in adults is not stated. Stable narrow complex IRREGULAR rhythms other than atrial fibrillation with RVR are not addressed.	Confirm sequence and scope.
U-2011-3	2011.2 Paramedic block (p.65)	Adult unstable line says 'Cardioversion: 100, 200, 300, 360 J'; the word 'unsynchronized' appears only for torsades. Synchronization for the other rhythms is not stated, and 'monophasic or biphasic equivalent' does not say how a biphasic device setting is chosen. AF with RVR is to be cardioverted at 200 J with an MCP order; a 'rapid ventricular rate' threshold is not defined.	Medical Director: state synchronized / unsynchronized by rhythm and device energy selection.
U-2011-4	2011.2 vs 8003 / 8001 (p.229)	2011 gives amiodarone for stable wide-complex tachycardia with no cautions. 8003 contraindications: pulmonary congestion, cardiogenic shock, hypotension (SBP less than 100), sensitivity. 8003 pediatric: 'Not indicated for stable wide complex tachycardia' (2011 lists no pediatric wide-complex treatment). Lidocaine is the backup 'if Amiodarone not available'; 8026 lists second / third degree heart block as a contraindication.	Confirm that monograph contraindications apply.
U-2011-5	2011.2 vs 8024 (p.272)	2011 says Ketamine 25 mg IV is the PREFERRED sedation before cardioversion; 8024 contraindications include suspected cardiac chest pain, hypertensive crisis and conditions where BP elevation could be harmful (acute MI, angina, aortic dissection); 1014.1.d (PDF p.41) says ketamine is not for suspected cardiac chest pain. Chest pain and MI are listed in 2011's own S&S and differential.	Medical Director: does the 'preferred' status override these contraindications?
U-2011-6	2011.2 vs 8029 (p.281)	Torsades: 8029 indication is 'in adult patient (with or without a pulse)'; 2011 uses adult markers only and gives no pediatric option. 8029 contraindications (any heart block except first degree; caution SBP less than 90; acute MI and abdominal pain are listed under precautions) are not stated in 2011. Stable vs unstable torsades is not defined beyond the general lists.	Confirm applicability and pediatric handling.
U-2011-7	2011.2 S&S vs Pearls (p.66)	'Unstable' is described as hypotension or altered mental status (S&S list) and in Pearls as hypotension, altered mental status, diaphoresis or cool/clammy skin. The pediatric criteria for unstable tachycardia are not stated separately. A single rate threshold (greater than 100 bpm) is given for all ages.	Confirm definition, especially for children.
U-2011-8	2011.2 Pediatric (p.66)	Pediatric adenosine: 'Repeat x 1' follows the 0.2 mg/kg dose (8001: 'May repeat x one'); whether the 0.1 mg/kg dose may be given again before escalating is not stated. 'Consider cardioversion' is listed beneath the adenosine bullets with 'If time permits' midazolam (2011: max 2.5 mg; 2010 pediatric pacing says max 2 mg). Pediatric = less than 16 years (1005.4) but dosing is by weight; no weight ranges are given.	Confirm sequencing.
U-2011-9	2011.2 Appendix A (p.322)	Appendix A (PDF p.322) lists new 2011 language: transport stable AF with RVR; cardiovert unstable AF with RVR by MCP order only; new Pearls. These are reflected in this checklist. This protocol's 'Last modified' date is May 16, 2026; the cover says the book was last updated 07/01/2026, Appendix A is effective Nov. 1, 2026 (see G-1).	Confirm edition in force.
U-2011-10	2011.2 EMT block (p.65)	{12 Lead ECG} (EMT) vs {12/15 Lead ECG} (Paramedic); both are bracketed agency options. 2010 uses the term 'EKG', 2011 uses 'ECG' — treated as the same test. Clinical Pearl about compensatory causes is Paramedic-level in the source layout only because the whole Pearls box is unlabelled; levels for Pearls are not printed.	Agency: which 12-lead option is adopted?

3002 Major Trauma

ID	Source location	Issue	Clarification needed
U-3002-1	3002.1 Tx Algorithm (p.71)	'Patients meeting criteria for transport to a Trauma Center are considered Load and Go' — the criteria are in 3018 Trauma Triage Guidelines, which was not reproduced here.	Add 3018 in a later batch or attach the triage criteria.
U-3002-2	3002.1 Consult vs 1002.3.b vs 3001 (p.15)	MIVT is defined as 'Mechanism, Injuries, Vital Signs and Time' in 1002.3.b (PDF p.15) but as 'Mechanism of Injury, Injuries, Vital signs, Treatment' in 3001 EMT block (PDF p.69). 3002 does not define it.	Confirm the meaning of 'T'.
U-3002-3	3002.1 Tx Algorithm (p.71)	Level reading: needle decompression is printed in the GREEN (AEMT) block, the EMT block says 'No additional orders at this level', and finger thoracostomy sits in the BLUE (Paramedic) block. Level information was read from the color bars of the rendered page (G-2).	Reviewer: confirm against the printed page and the agency's scope-of-practice documents.
U-3002-4	3002.1 scope (p.71)	3002 contains only M.A.R.C.H. priorities, chest-injury considerations and a Consult line. It does not state methods or orders for hemorrhage control (3015), spinal motion restriction (3017), IV / fluids, pain (1014), head injury (3013), trauma arrest (3004), destination (3018 / 3020) or the EMT trauma items printed in 3001. 'Consider' M.A.R.C.H. is discretionary wording.	Do not treat this checklist as a complete trauma checklist; add related protocols in later batches.
U-3002-5	3002.1 Paramedic block (p.71)	Finger thoracostomy is a bracketed { } option 'in accordance to their specific protocols' — no protocol text, indications or training standard is in this document. Appendix A (PDF pp.322-323) lists it as newly added (also at 1008.1, not reviewed).	Agency: adopted? Provide the specific protocol.
U-3002-6	3002.1 AEMT block (p.71)	Needle decompression: 'as indicated' — the signs of tension pneumothorax are not listed in 3002. Catheter length is printed as 3 ¼ inch with '14-gauge or larger'. No pediatric catheter size is given; the under-8-years rule limits the site only. Right main stem intubation caution refers to intubation, which is not in this protocol.	Medical Director: define indications and pediatric equipment.
U-3002-7	3002.1 Tx Algorithm (p.71)	Oxygen: 'oxygenate the patient with 100% O2' in 3002 vs SpO2-titrated oxygen (e.g., titrate to 94%) in other protocols reviewed (2008, 4017). 3002 does not say when to stop or reduce oxygen.	Confirm intended oxygen strategy for major trauma.

4002 Allergic Reaction / Anaphylaxis

ID	Source location	Issue	Clarification needed
U-4002-1	4002.2 Consult vs Appendix A vs 8002 / 8023 (p.120)	4002 Consult (p.120): 'EMT needs MCP orders to administer breathing treatments.' Appendix A (p.322): '4002.2 EMT — Removed requirement for the EMT to contact MCP for nebulized Albuterol and Ipratropium' and '4002.2 AEMT — Removed line stating no orders needed for the AEMT to administer nebulized meds, due to EMT change for orders'. Appendix A (p.323): 8002 and 8023 Medical Control — 'Removed requirement for EMTs to contact medical control'; both monographs now print Medical Control 'Adults: No / Pediatrics: No'. The 4002 Consult text was not updated.	Medical Director: is an MCP order required for EMT nebulized treatments? (Checklist item left untagged as a CONFLICT.)
U-4002-2	4002.2 (p.119)	Repeat epinephrine: 'may repeat ... in 10 minutes' — the number of repeats and maximum total dose are not stated in 4002, 8017 or 8018. At EMR level the repeat line is a diamond; at AEMT level the repeat is printed with no diamond; the Consult says 'The EMR and EMT need MCP orders to administer repeat epinephrine'. 8017 / 8018 Medical Control: initial dose No; follow-up dosing for AEMT and Paramedic No; repeat doses by EMR / EMTs Yes.	Confirm maximum number of epinephrine doses.
U-4002-3	4002.2 Pediatric Considerations vs 8017 / 1005.4 (p.257)	The protocol 'lists those patients under 15 kg as pediatric' and dosing is by weight; 1005.4 defines pediatric as less than 16 years. 8017 pediatric dosing says 'If greater than 30 kg'; 4002 says 'equal to or greater than 30 kg'. Patients weighing exactly 30 kg are treated differently by the two documents.	Confirm the 30 kg wording.
U-4002-4	4002.2 EMT block (p.119)	The EMT syringe line prints 'Epinephrine (1:1,000) 0.5 IM' without a unit (0.5 mg appears in the repeat line). The EMT syringe option is bracketed { } and requires Medical Director authorization and training; the diamond appears on the repeat lines only.	Agency: adopted? Confirm unit.
U-4002-5	4002.2 AEMT block vs 8013 (p.249)	Diphenhydramine: 4002 says 'IM or IV'; 8013 says 'IM or slow IV' (1005.4 defines slow IV as over 2 minutes unless otherwise specified). 8013 Medical Control: 'No, for the Paramedic. Yes, for the AEMT when treating Extrapyramidal Reactions' — AEMT status for allergic reaction is not stated.	Confirm route and AEMT order requirement.
U-4002-6	4002.2 AEMT block (p.119)	Hypotension: adult 'IV fluid to maintain adequate BP' has no volume, fluid type or BP definition; pediatric is 20 ml/kg IV. IV access instructions and 8036 fluid monograph are not reproduced here.	Medical Director clarification.
U-4002-7	4002.2 Paramedic block vs 8019 (p.261)	Epinephrine (1:10,000) 0.1 mg slow IV every 3 minutes up to 0.5 mg appears with no adult / pediatric marker, no pediatric dose and no 'slow' duration. 8019 lists Paramedic indications as arrest rhythms and pediatric bradycardia only, with no anaphylaxis / hypotension use.	Confirm indication, pediatric dose and push time.
U-4002-8	4002.1 / 4002.2 (p.119)	The Treatment Algorithm starts 'If allergic reaction:' and gives epinephrine; 4002.1.a-b describe epinephrine as the mainstay for anaphylaxis and especially with airway edema, hypotension or multiple body systems. No criteria are given for anaphylaxis vs allergic reaction, or for when epinephrine is withheld.	Medical Director: severity criteria.
U-4002-9	4002.2 AEMT / Paramedic blocks (p.119)	Intubation: 1008 Advanced Airway Management is not reproduced; 8031 says the AEMT should intubate if apneic. 4002 gives no tube size, device or confirmation steps. Solu-Medrol 'after all other applicable first-line medications' is not defined; 8030 says do not start an IV only to give it.	Add 1008 in a later batch if needed.
U-4002-10	Appendix A (p.322)	4002 'Last modified' is May 16, 2026; the cover says the book was last updated 07/01/2026 and Appendix A is effective Nov. 1, 2026 (G-1). Monograph level columns were not read (G-5).	Confirm edition in force.

4017 Stroke

ID	Source location	Issue	Clarification needed
U-4017-1	4017.2 EMR block (p.151)	Oxygen: 'SpO2 less than 94% ... titrated to 94%' and 'SpO2 greater than 94% ... should not get any oxygen'. A reading of exactly 94% is not addressed (2008 uses '94% or higher').	Confirm the 94% case.
U-4017-2	4017.2 EMR block (p.151)	NRB mask for 'respiratory distress with pale, moist skin and altered mental status' vs the SpO2 rule (no oxygen above 94%). Which governs is not stated.	Medical Director clarification.
U-4017-3	4017.2 EMR block (p.151)	'Signs of cerebral herniation' are not defined in 4017. Adult rate 'approximately 20 times per minute'; pediatric 'a rate of ten faster than normal respiratory rate' (10 breaths per minute faster?). EtCO2 target (approximately 30 mmHg) is a bracketed { } option.	Define herniation signs and the pediatric rate.
U-4017-4	4017.2 EMT block vs 4017.3.c (p.152)	'Comprehensive Stroke Centers or Thrombectomy Capable' are defined together (4017.3.c), and Comprehensive centers are named. The rule that suspected HEMORRHAGIC stroke should not go to a 'Thrombectomy Capable Facility' does not say whether it applies to the Comprehensive centers (Miami Valley, Kettering), which share the 24/7 endovascular capability.	Medical Director: clarify destination for suspected hemorrhagic stroke.
U-4017-5	4017.1.a / 4017.2 (p.151)	'Closest appropriate Stroke Center' is not defined; only the Comprehensive / Thrombectomy centers are named (4017.3.c). Telemedicine and Primary Stroke Centers are described but not listed (hospital capability chart 7012 not reproduced). 4017.1.a says transport when less than 24 hours; 4017.2 allows exceptions for LVO indicators and thrombolytic considerations; 4017.1.e lets agencies base destination on proximity and capabilities.	Agency: provide destination matrix.
U-4017-6	4017.2 EMT block (p.152)	Glucose: 4017 gives only a conditional ('If glucose is less than 60, or strong suspicion of hypoglycemia'); the instruction to check a glucose is not stated in 4017 and units are not given. The 1005.4 EMR block ('Perform blood glucose check') is cited as the source of the check. 4008 is not reproduced here.	Confirm intended glucose step and add 4008 in a later batch.
U-4017-7	4017.2 EMT block (p.152)	Thrombolytic-contraindication list uses source spellings 'Apixiban' and 'Abigatran'; the visible page prints 'contraindications' where the text layer reads 'contradictions'. The list is a 'consider' item; no step says what to do with a positive finding other than consider Comprehensive / Thrombectomy transport.	Reviewer: confirm names and intent.
U-4017-8	4017.2 Pearls (p.152)	'Arrange for transport a historian with patient' is garbled; it is not clear whether the historian is to ride with the patient or follow, or how consent for treatment is to be handled otherwise.	Confirm wording.

ID	Source location	Issue	Clarification needed
U-4017-9	4017.2 / Appendix A (p.322)	4017 'Last modified' is Jan. 21, 2024 and does not appear in Appendix A's 2026-2028 changes. Optional { } items: alternative stroke screen, EtCO2 ventilation target. 'Increased ICP' signs for the bed-flat rule are not defined.	Agency: adopted options; define ICP signs.

AUDIT SUMMARY

Automated source audit at build: 167 checklist items in this file - every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures; whole-document audit 335 items, 0 failures); 181 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 24 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

SOURCE-VERIFICATION TABLES

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

One row per checklist line. 'Traced' = the cited section and PDF page were located and the item's key phrases and numbers were machine-matched to that page; levels were read from the rendered color bars. Human review is still pending. Level column: lowest level at which the source lists the item (cumulative).

2010 Bradycardia — PDF p.63 (+ monographs 8005 p.233, 8006/8007 pp.235-237, 8019 p.261, 8024 pp.271-272, 8031 p.285, 8042 p.307)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2010-1.01	Bradycardia is any heart rate less than 60 bpm.	2010.1.a (p.63)	ALL	-	Traced; review pending
2010-1.02	Non-symptomatic bradycardia may be a normal finding in...	2010.1.b, 2010.1.d (p.63)	ALL	-	Traced; review pending
2010-1.03	Signs & symptoms: heart rate less than 60 bpm; syncope...	2010.2 Signs & Symptoms (p.63)	EMR+	-	Traced; review pending
2010-1.04	Unstable bradycardia is listed with: hypotension; altered...	2010.2 Signs & Symptoms (p.63)	EMR+	-	Traced; review pending
2010-1.05	Listed differential: acute myocardial infarction; hypoxia...	2010.2 Differential Diagnosis (p.63)	ALL	-	Traced; review pending
2010-1.06	PEDIATRIC: with adequate perfusion, monitor vital signs and...	2010.2 Pediatric Considerations (p.63)	ALL	-	Traced; review pending
2010-2.01	Assess the patient and determine medical history.	2010.1.c (p.63)	ALL	-	Traced; review pending
2010-2.02	Obtain {12 Lead EKG}.	2010.2 Tx Algorithm, EMT block (p.63)	EMT+	-	Traced; review pending
2010-2.03	Obtain and interpret {12/15 Lead EKG}.	2010.2 Tx Algorithm, Paramedic block (p.63)	PM	EKG wording differs by level block (see U-2010-9).	Traced; review pending
2010-2.04	PEDIATRIC less than 2 years old: look for signs and symptoms of...	2010.2 Tx Algorithm, EMT block (p.63)	EMT+	-	Traced; review pending
2010-2.05	Required assessment elements and vital-sign frequency for...	-		-	NOT SPECIFIED — VERIFY
2010-3.01	Administer oxygen as indicated.	2010.2 Tx Algorithm, EMR block (p.63)	EMR+	-	Traced; review pending
2010-3.02	PEDIATRIC less than 2 years old: secure the airway and...	2010.2 Tx Algorithm, EMT block (p.63)	EMT+	-	Traced; review pending
2010-3.03	No additional orders at this level (source).	2010.2 Tx Algorithm, AEMT block (p.63)	AEMT +	-	Traced; review pending
2010-3.04	Wide complex bradycardia patients should spark consideration of...	2010.2 Tx Algorithm, Paramedic block (p.63)	PM	-	Traced; review pending
2010-3.05	ADULT, MCP ORDER (paramedic consults): administer BOTH Calcium...	2010.2 Tx Algorithm, Paramedic block; Consult (p.63); 8006 Adult Dosing (p.235); 8042 Adult Dosing (p.307)	PM	Source states no dialysis requirement; 8006 lists bradycardia only for renal dialysis patients (see U-2010-1).	Traced; review pending
2010-3.06	Flush well between these medications. It is critical that these...	2010.2 Tx Algorithm, Paramedic block (p.63); 8006 Precautions (p.235)	PM	-	Traced; review pending
2010-3.07	Pediatric calcium / sodium bicarbonate dosing for bradycardia...	-		-	NOT SPECIFIED — VERIFY
2010-3.08	ADULT: consider Atropine 1 mg IV, up to a total of 3 mg.	2010.2 Tx Algorithm, Paramedic block (p.63); 8005 Adult Dosing (p.233)	PM	Interval between adult doses is not stated in 2010 or 8005 (see U-2010-2). Source lists this under 'poor perfusion in adults and pediatrics' with an adult marker.	Traced; review pending
2010-3.09	Repeat interval between adult atropine doses: NOT SPECIFIED IN...	-		-	NOT SPECIFIED — VERIFY
2010-3.10	If treatments are ineffective, begin pacing.	2010.2 Tx Algorithm, Paramedic block (p.63)	PM	-	Traced; review pending
2010-3.11	ADULT pacing, if time permits: Ketamine 25 mg IV (preferred...	2010.2 Tx Algorithm, Paramedic block (p.63); 8024 Adult Dosing (p.271); 8031 Adult Dosing (p.285)	PM	8024 lists suspected cardiac chest pain and acute MI as ketamine contraindications (see U-2010-6).	Traced; review pending
2010-3.12	DO NOT reduce Ketamine or Midazolam doses by half for patients...	2010.2 Tx Algorithm, Paramedic block (p.63); 8024 Adult Dosing (p.271)	PM	-	Traced; review pending
2010-3.13	ADULT pacing settings: set at 70 BPM, 20 mA and increase until...	2010.2 Tx Algorithm, Paramedic block (p.63)	PM	No maximum current is stated for adults (see U-2010-7).	Traced; review pending
2010-3.14	PEDIATRIC: Epinephrine (1:10,000) 0.01 mg/kg IV, repeat every 5...	2010.2 Tx Algorithm, Paramedic block (p.63); 8019 Pediatric Dosing (p.261)	PM	8019: repeat every 3-5 minutes, max single dose 1 mg (see U-2010-4).	Traced; review pending
2010-3.15	PEDIATRIC, if AV block: consider Atropine 0.02 mg/kg IV...	2010.2 Tx Algorithm, Paramedic block (p.63); 8005 Pediatric Dosing (p.233)	PM	8005 precaution: paradoxical bradycardia at doses less than 0.5 mg (see U-2010-3).	Traced; review pending
2010-3.16	PEDIATRIC, if AV block, consider pacing: pediatric electrodes...	2010.2 Tx Algorithm, Paramedic block (p.63); 8031 Pediatric Dosing (p.285)	PM	8031 gives a 2.5 mg maximum for this use (see U-2010-5).	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2010-3.17	ATROPINE (8005) contraindications: tachycardia...	8005 Contraindications / Precautions (p.233)	PM	-	Traced; review pending
2010-3.18	CALCIUM (8006): contraindications none in the emergency...	8006 Contraindications / Precautions (p.235)	PM	-	Traced; review pending
2010-3.19	SODIUM BICARBONATE (8042) precautions: metabolic alkalosis...	8042 Contraindications / Precautions (p.307)	PM	-	Traced; review pending
2010-3.20	KETAMINE (8024) contraindications: suspected cardiac chest...	8024 Contraindications / Precautions (p.272)	PM	See U-2010-6.	Traced; review pending
2010-3.21	MIDAZOLAM (8031): contraindication respiratory distress; can...	8031 Contraindications / Precautions (p.285)	PM	-	Traced; review pending
2010-4.01	For adequate perfusion: observe and monitor vital signs.	2010.2 Tx Algorithm, EMR block (p.63)	EMR+	-	Traced; review pending
2010-4.02	PEDIATRIC less than 2 years old: if heart rate and perfusion do...	2010.2 Tx Algorithm, EMT block (p.63)	EMT+	No heart-rate threshold is stated for starting CPR (see U-2010-8).	Traced; review pending
2010-4.03	Reassessment interval, response-to-treatment criteria and...	-		-	NOT SPECIFIED — VERIFY
2010-5.01	Call for transport immediately.	2010.2 Tx Algorithm, EMR block (p.63)	EMR+	-	Traced; review pending
2010-5.02	Transport immediately unless ALS intercept is less than 5...	2010.2 Tx Algorithm, EMT block (p.63)	EMT+	-	Traced; review pending
2010-5.03	Transmit the {12 Lead EKG} and call the receiving facility.	2010.2 Tx Algorithm, EMT block (p.63)	EMT+	-	Traced; review pending
2010-5.04	Paramedic consults MCP for administration of Calcium Chloride...	2010.2 Consult (p.63)	PM	-	Traced; review pending
2010-5.05	Destination / receiving-facility selection for bradycardia and...	-		-	NOT SPECIFIED — VERIFY
2010-6.01	Documentation / closeout elements for bradycardia: NOT...	-		-	NOT SPECIFIED — VERIFY

2011 Tachycardia — PDF pp.65-66 (+ monographs 8001 p.225, 8003 p.229, 8024 pp.271-272, 8026 p.275, 8029 p.281, 8031 p.285)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2011-1.01	Tachycardia is any heart rate greater than 100 bpm. Treat...	2011.1.a, 2011.1.c (p.65)	ALL	-	Traced; review pending
2011-1.02	Signs & symptoms: heart rate greater than 100 bpm; dizziness...	2011.2 Signs & Symptoms (p.65)	EMR+	-	Traced; review pending
2011-1.03	Unstable tachycardia is listed with: hypotension; altered...	2011.2 Signs & Symptoms (p.65)	EMR+	-	Traced; review pending
2011-1.04	Hemodynamically unstable patients will typically present with...	2011.2 Clinical Pearls (p.66)	ALL	Differs from the Signs & Symptoms list (see U-2011-7).	Traced; review pending
2011-1.05	Listed differential: myocardial infarction; electrolyte...	2011.2 Differential Diagnosis (p.65)	ALL	-	Traced; review pending
2011-1.06	PEDIATRIC: with adequate perfusion, monitor vital signs and...	2011.2 Pediatric Considerations (p.65)	ALL	-	Traced; review pending
2011-2.01	Assess the patient and determine medical history.	2011.1.b (p.65)	ALL	-	Traced; review pending
2011-2.02	Obtain {12 Lead ECG}.	2011.2 Tx Algorithm, EMT block (p.65)	EMT+	-	Traced; review pending
2011-2.03	Obtain and interpret {12/15 Lead ECG}.	2011.2 Tx Algorithm, Paramedic block (p.65)	PM	-	Traced; review pending
2011-2.04	Rule out or treat compensatory and physiological causes prior...	2011.2 Clinical Pearls (p.66)	ALL	-	Traced; review pending
2011-2.05	Required assessment elements, vital-sign frequency and a rate...	-		-	NOT SPECIFIED — VERIFY
2011-3.01	Administer oxygen as indicated.	2011.2 Tx Algorithm, EMR block (p.65)	EMR+	-	Traced; review pending
2011-3.02	No additional orders at this level (source).	2011.2 Tx Algorithm, AEMT block (p.65)	AEMT +	-	Traced; review pending
2011-3.03	Stable, narrow complex, regular: vagal maneuvers.	2011.2 Tx Algorithm, Paramedic block (p.65)	PM	-	Traced; review pending
2011-3.04	Stable, narrow complex, regular: Adenosine 6 mg rapid IVP...	2011.2 Tx Algorithm, Paramedic block (p.65); 8001 Adult Dosing (p.225)	PM	See U-2011-1 (flush volume, repeat count).	Traced; review pending
2011-3.05	If PSVT history and the patient advises it takes 12 mg of...	2011.2 Clinical Pearls (p.66); 8001 Adult Dosing (p.225)	PM	-	Traced; review pending
2011-3.06	Stable, atrial fibrillation with rapid ventricular rate (RVR)...	2011.2 Tx Algorithm, Paramedic block (p.65)	PM	-	Traced; review pending
2011-3.07	Stable, wide complex, regular or irregular: Amiodarone 150 mg...	2011.2 Tx Algorithm, Paramedic block (p.65); 8003 Adult Dosing (p.229)	PM	8003 contraindications not stated in 2011 (see U-2011-4).	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2011-3.08	If Amiodarone is not available, use Lidocaine 150 mg IV/IO.	2011.2 Tx Algorithm, Paramedic block (p.65); 8026 Adult Dosing (p.275)	PM	-	Traced; review pending
2011-3.09	Consider a sedative/analgesic prior to cardioversion: Ketamine...	2011.2 Tx Algorithm, Paramedic block (p.65); 8024 Adult Dosing (p.271); 8031 Adult Dosing (p.285)	PM	8024 contraindications (see U-2011-5).	Traced; review pending
2011-3.10	DO NOT reduce Ketamine or Midazolam doses by half for patients...	2011.2 Tx Algorithm, Paramedic block (p.65)	PM	-	Traced; review pending
2011-3.11	Cardioversion: 100, 200, 300, 360 J for monophasic or biphasic...	2011.2 Tx Algorithm, Paramedic block (p.65)	PM	Synchronization is not stated on this line (see U-2011-3).	Traced; review pending
2011-3.12	MCP ORDER: if atrial fibrillation with RVR is unstable...	2011.2 Tx Algorithm, Paramedic block (p.65); 2011.2 Consult (p.66)	PM	-	Traced; review pending
2011-3.13	Paramedics should NOT cardiovert: patients without hemodynamic...	2011.2 Clinical Pearls (p.66)	PM	-	Traced; review pending
2011-3.14	If stable: Magnesium Sulfate 2 gm infused with macro-drip...	2011.2 Tx Algorithm, Paramedic block (p.65); 8029 Adult Dosing (p.281)	PM	-	Traced; review pending
2011-3.15	If unstable: UNSYNCHRONIZED cardioversion 100, 200, 300, 360 J...	2011.2 Tx Algorithm, Paramedic block (p.65)	PM	-	Traced; review pending
2011-3.16	Stable pediatrics: vagal maneuvers (blowing through a straw or...	2011.2 Tx Algorithm, Paramedic block (p.66)	PM	-	Traced; review pending
2011-3.17	Unstable pediatrics: Adenosine 0.1 mg/kg rapid IVP (max dose 6...	2011.2 Tx Algorithm, Paramedic block (p.66); 8001 Pediatric Dosing (p.225)	PM	-	Traced; review pending
2011-3.18	If no response: Adenosine 0.2 mg/kg rapid IVP (max dose 12 mg)...	2011.2 Tx Algorithm, Paramedic block (p.66); 8001 Pediatric Dosing (p.225)	PM	-	Traced; review pending
2011-3.19	Consider cardioversion. If time permits: Midazolam 0.1 mg/kg...	2011.2 Tx Algorithm, Paramedic block (p.66); 8031 Pediatric Dosing (p.285)	PM	-	Traced; review pending
2011-3.20	Pediatric cardioversion 1 J/kg. If no response, repeat...	2011.2 Tx Algorithm, Paramedic block (p.66)	PM	-	Traced; review pending
2011-3.21	Pediatric treatment for wide complex tachycardia, pediatric...	-		-	NOT SPECIFIED — VERIFY
2011-3.22	ADENOSINE (8001) contraindications: second or third degree AV...	8001 Contraindications / Precautions (p.225)	PM	-	Traced; review pending
2011-3.23	AMIODARONE (8003) contraindications: pulmonary congestion...	8003 Contraindications / Precautions (p.229)	PM	-	Traced; review pending
2011-3.24	LIDOCAINE (8026) contraindications: hypersensitivity; second or...	8026 Contraindications / Precautions (p.275)	PM	-	Traced; review pending
2011-3.25	MAGNESIUM SULFATE (8029) contraindications: hypersensitivity...	8029 Contraindications / Precautions (p.281)	PM	-	Traced; review pending
2011-3.26	KETAMINE (8024) contraindications: suspected cardiac chest...	8024 Contraindications (p.272); 8031 Contraindications / Precautions (p.285)	PM	See U-2011-5.	Traced; review pending
2011-4.01	Reassessment interval, response-to-treatment criteria...	-		-	NOT SPECIFIED — VERIFY
2011-5.01	Call for transport immediately.	2011.2 Tx Algorithm, EMR block (p.65)	EMR+	-	Traced; review pending
2011-5.02	Transport immediately unless ALS intercept is less than 5...	2011.2 Tx Algorithm, EMT block (p.65)	EMT+	-	Traced; review pending
2011-5.03	Transmit the {12 Lead ECG} and call the receiving facility.	2011.2 Tx Algorithm, EMT block (p.65)	EMT+	-	Traced; review pending
2011-5.04	Paramedics must consult MCP for orders to cardiovert unstable...	2011.2 Consult (p.66)	PM	-	Traced; review pending
2011-5.05	Destination / receiving-facility selection for tachycardia and...	-		-	NOT SPECIFIED — VERIFY
2011-6.01	Documentation / closeout elements for tachycardia: NOT...	-		-	NOT SPECIFIED — VERIFY

3002 Major Trauma — PDF p.71 (+ 1002 p.15, 1005 p.21)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
3002-1.01	Sign / symptom listed: significant injuries or life threats.	3002.1 Signs & Symptoms (p.71)	EMR+	-	Traced; review pending
3002-1.02	Patients meeting criteria for transport to a Trauma Center are...	3002.1 Tx Algorithm, EMR block (p.71)	EMR+	The Trauma Center criteria are not in 3002 (see U-3002-1).	Traced; review pending
3002-1.03	Consider the trauma care mnemonic M.A.R.C.H. to establish...	3002.1 Tx Algorithm, EMR block (p.71)	EMR+	-	Traced; review pending
3002-1.04	Pediatric Considerations: None. Differential Diagnosis: None.	3002.1 Assessment (p.71)	ALL	-	Traced; review pending
3002-2.01	M - Massive Hemorrhage: assess for and address significant...	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
3002-2.02	A - Airway: assess for a patent airway.	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-2.03	R - Respirations: assess respiratory rate, quality and effort.	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-2.04	C - Circulation: perform a head-to-toe sweep for additional...	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-2.05	Required primary / secondary survey elements, vital signs, GCS...	-		-	NOT SPECIFIED — VERIFY
3002-3.01	A - Airway: ensure a patent airway using adjuncts and...	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-3.02	R - Respirations: oxygenate the patient with 100% O2.	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-3.03	H - Hypothermia: prophylactically treat the patient for...	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-3.04	Open pneumothorax: cover the wound with an occlusive dressing...	3002.1 Other trauma care considerations (p.71)	EMR+	-	Traced; review pending
3002-3.05	Tension pneumothorax: lift one side of any occlusive dressing.	3002.1 Other trauma care considerations (p.71)	EMR+	-	Traced; review pending
3002-3.06	Flail chest: stabilize immediately with a gloved hand, then...	3002.1 Tx Algorithm, EMR block (p.71)	EMR+	-	Traced; review pending
3002-3.07	No additional orders at this level (source).	3002.1 Tx Algorithm, EMT block (p.71)	EMT+	-	Traced; review pending
3002-3.08	Use caution not to confuse right main stem intubation for a...	3002.1 Tx Algorithm, AEMT block (p.71)	AEMT +	Level read from the green color bar (see U-3002-3).	Traced; review pending
3002-3.09	Perform needle decompression as indicated: decompress the chest...	3002.1 Tx Algorithm, AEMT block (p.71)	AEMT +	Indications not defined (see U-3002-6).	Traced; review pending
3002-3.10	Site options: fourth or fifth intercostal space in the...	3002.1 Tx Algorithm, AEMT block (p.71)	AEMT +	-	Traced; review pending
3002-3.11	PEDIATRIC less than 8 years old: decompression site choice will...	3002.1 Tx Algorithm, AEMT block (p.71)	AEMT +	-	Traced; review pending
3002-3.12	{With defined and specific indications, paramedics who have...	3002.1 Tx Algorithm, Paramedic block (p.71)	PM	Agency option; specific protocol is not in this document (see U-3002-5).	Traced; review pending
3002-3.13	For a pregnant patient in arrest, consider the need for manual...	3002.1 Clinical Pearls (p.71)	ALL	-	Traced; review pending
3002-3.14	Hemorrhage-control methods, IV / fluid orders, spinal motion...	-		-	NOT SPECIFIED — VERIFY
3002-4.01	C - Circulation: double check all bleeding interventions.	3002.1 M.A.R.C.H. (p.71)	EMR+	-	Traced; review pending
3002-4.02	Reassessment interval, response criteria and worsening pathway...	-		-	NOT SPECIFIED — VERIFY
3002-5.01	Patients meeting criteria for transport to a Trauma Center are...	3002.1 Tx Algorithm, EMR block (p.71)	EMR+	Criteria are not in 3002 (see U-3002-1).	Traced; review pending
3002-5.02	Consult: contact Medical Control and advise them of patient...	3002.1 Consult (p.71); 1002.3.b (p.15)	ALL	MIVT is defined differently in 1002.3.b and 3001 (see U-3002-2).	Traced; review pending
3002-5.03	Destination / Trauma Center selection, notification system use...	-		-	NOT SPECIFIED — VERIFY
3002-6.01	Documentation / closeout elements for major trauma: NOT...	-		-	NOT SPECIFIED — VERIFY

4002 Allergic Reaction / Anaphylaxis — PDF pp.119-120 (+ monographs 8002 p.227, 8013 p.249, 8017 p.257, 8018 p.259, 8019 p.261, 8023 p.269, 8026 p.275, 8030 p.283; Appendix A p.322)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4002-1.01	Epinephrine is the mainstay of anaphylaxis in allergic reaction...	4002.1.a (p.119)	ALL	-	Traced; review pending
4002-1.02	Epinephrine is particularly important in cases of any airway...	4002.1.b (p.119)	ALL	-	Traced; review pending
4002-1.03	Advanced age is not a contraindication to epinephrine.	4002.1.c (p.119)	ALL	-	Traced; review pending
4002-1.04	Signs & symptoms: itching; hoarseness or stridor; wheezing...	4002.2 Signs & Symptoms (p.119)	EMR+	-	Traced; review pending
4002-1.05	Listed differential: rash only; shock (vascular effect)...	4002.2 Differential Diagnosis (p.119)	ALL	-	Traced; review pending
4002-1.06	PEDIATRIC: epinephrine is dosed based on weight, not age.	4002.2 Pediatric Considerations (p.119)	ALL	-	Traced; review pending
4002-1.07	PEDIATRIC: the protocol lists patients under 15 kg as...	4002.2 Pediatric Considerations (p.119)	ALL	See U-4002-3.	Traced; review pending
4002-2.01	Required assessment steps, vital signs, history (allergen /...	-		-	NOT SPECIFIED — VERIFY
4002-3.01	Provide O2 as needed.	4002.2 Tx Algorithm, EMR block (p.119)	EMR+	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4002-3.02	If allergic reaction and weight is equal to or greater than 30...	4002.2 Tx Algorithm, EMR block (p.119); 8017 Adult / Pediatric Dosing (p.257)	EMR+	8017: initial dose needs no order at any level (Medical Control: No).	Traced; review pending
4002-3.03	PEDIATRIC less than 15 kg: EpiPen Jr.	4002.2 Tx Algorithm, EMR block (p.119); 8017 Pediatric Dosing (p.257)	EMR+	-	Traced; review pending
4002-3.04	PEDIATRIC equal to or greater than 15 kg and less than 30 kg...	4002.2 Tx Algorithm, EMR block (p.119); 8017 Pediatric Dosing (p.257)	EMR+	-	Traced; review pending
4002-3.05	If applicable, apply an ice pack.	4002.2 Tx Algorithm, EMR block (p.119)	EMR+	-	Traced; review pending
4002-3.06	If the patient develops wheezing, assist with their prescribed...	4002.2 Tx Algorithm, EMT block (p.119); 8002 Adult Dosing (p.227); 8023 Adult Dosing (p.269)	EMT+	CONFLICT on whether the EMT needs an MCP order (see U-4002-1).	Traced; review pending
4002-3.07	{If allergic reaction and an absence of Epi-pens in the drug...	4002.2 Tx Algorithm, EMT block (p.119); 8018 Indications (p.259); 4002.2 Clinical Pearls (p.120)	EMT+	-	Traced; review pending
4002-3.08	{Epinephrine (1:1,000) IM: equal to or greater than 30 kg 0.5...	4002.2 Tx Algorithm, EMT block (p.119); 8018 Adult / Pediatric Dosing (p.259)	EMT+	Source prints '0.5 IM' without the unit for the 30 kg line (see U-4002-4).	Traced; review pending
4002-3.09	If an allergic reaction: equal to or greater than 30 kg, give...	4002.2 Tx Algorithm, AEMT block (p.119); 8018 Adult Dosing (p.259)	AEMT +	-	Traced; review pending
4002-3.10	PEDIATRIC equal to or greater than 15 kg and less than 30 kg...	4002.2 Tx Algorithm, AEMT block (p.119); 8018 Pediatric Dosing (p.259)	AEMT +	-	Traced; review pending
4002-3.11	If apneic, intubate, possibly with smaller than normal ET tube.	4002.2 Tx Algorithm, AEMT block (p.119)	AEMT +	1008 Advanced Airway Management not reproduced (G-3).	Traced; review pending
4002-3.12	If the patient is intubated, Albuterol 2.5 mg by nebulizer into...	4002.2 Tx Algorithm, AEMT block (p.119)	AEMT +	-	Traced; review pending
4002-3.13	If hypotensive: ADULT IV fluid to maintain adequate BP...	4002.2 Tx Algorithm, AEMT block (p.119)	AEMT +	Adult volume and 'adequate BP' not defined (see U-4002-6).	Traced; review pending
4002-3.14	ADULT: Diphenhydramine 50 mg IM or IV. PEDIATRIC...	4002.2 Tx Algorithm, AEMT block (p.119); 8013 Adult / Pediatric Dosing (p.249)	AEMT +	8013 says 'IM or slow IV' (see U-4002-5).	Traced; review pending
4002-3.15	If the patient is deteriorating or unresponsive, consider early...	4002.2 Tx Algorithm, Paramedic block (p.120)	PM	-	Traced; review pending
4002-3.16	If a conscious patient requires intubation — ADULT: Lidocaine...	4002.2 Tx Algorithm, Paramedic block (p.120); 8026 Adult / Pediatric Dosing (p.275)	PM	-	Traced; review pending
4002-3.17	If the patient remains hypotensive after IV fluid: Epinephrine...	4002.2 Tx Algorithm, Paramedic block (p.120)	PM	Not in the 8019 indications; no pediatric dose given (see U-4002-7).	Traced; review pending
4002-3.18	Solu-Medrol: ADULT 125 mg IV. PEDIATRIC 2 mg/kg IV, max dose...	4002.2 Tx Algorithm, Paramedic block (p.120); 8030 Adult / Pediatric Dosing (p.283)	PM	-	Traced; review pending
4002-3.19	Solu-Medrol is given to all patients treated within the...	4002.2 Clinical Pearls (p.120); 8030 Precautions (p.283)	PM	-	Traced; review pending
4002-3.20	Pearl: no significant change in patient condition in the field...	4002.2 Clinical Pearls (p.120)	ALL	-	Traced; review pending
4002-3.21	EPINEPHRINE (8017, 8018, 8019): contraindications none in the...	8017 Contraindications / Precautions (p.257); 8018 Contraindications / Precautions (p.259); 8019 Contraindications / Precautions (p.261)	ALL	-	Traced; review pending
4002-3.22	DIPHENHYDRAMINE (8013): contraindications none in the emergency...	8013 Contraindications / Precautions (p.249)	AEMT +	-	Traced; review pending
4002-3.23	ALBUTEROL (8002): prior hypersensitive reaction to albuterol...	8002 Contraindications (p.227); 8023 Contraindications / Precautions (p.269)	EMT+	-	Traced; review pending
4002-3.24	LIDOCAINE (8026): hypersensitivity; second or third degree...	8026 Contraindications / Precautions (p.275)	PM	-	Traced; review pending
4002-3.25	SOLU-MEDROL (8030): contraindications none in emergency...	8030 Contraindications / Precautions (p.283)	PM	-	Traced; review pending
4002-4.01	If symptoms persist, may repeat Epinephrine (adult and...	4002.2 Tx Algorithm, EMR block (p.119); 4002.2 Consult (p.120); 8017 Medical Control (p.257)	EMR+	-	Traced; review pending
4002-4.02	{EMT, syringe option: may repeat Epinephrine (1:1,000) 0.5 mg...	4002.2 Tx Algorithm, EMT block (p.119); 4002.2 Consult (p.120)	EMT+	-	Traced; review pending
4002-4.03	EMT: may repeat Albuterol 2.5 mg nebulized two times.	4002.2 Tx Algorithm, EMT block (p.119); 8002 Adult Dosing (p.227)	EMT+	Order requirement conflict (see U-4002-1).	Traced; review pending
4002-4.04	AEMT / Paramedic: may repeat Epinephrine (1:1,000) 0.5 mg IM...	4002.2 Tx Algorithm, AEMT block (p.119); 8017 Medical Control (p.257)	AEMT +	Maximum number of doses not stated (see U-4002-2).	Traced; review pending
4002-4.05	Reassessment interval, response-to-treatment criteria, biphasic...	-		-	NOT SPECIFIED — VERIFY

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4002-5.01	Call for transport.	4002.2 Tx Algorithm, EMR block (p.119)	EMR+	-	Traced; review pending
4002-5.02	Destination, notification, MCP contact requirements other than...	-		-	NOT SPECIFIED — VERIFY
4002-6.01	Documentation / closeout elements for allergic reaction /...	-		-	NOT SPECIFIED — VERIFY

4017 Stroke — PDF pp.151-152 (+ 1005 p.22)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4017-1.01	If one or more signs of the Cincinnati Prehospital Stroke Scale...	4017.1.a (p.151)	ALL	'Closest appropriate' is not defined (see U-4017-5).	Traced; review pending
4017-1.02	In addition to the CPSS, screen patients for possible large...	4017.1.b (p.151)	ALL	-	Traced; review pending
4017-1.03	If greater than 24 hours since last known well, consider...	4017.1.c (p.151)	ALL	-	Traced; review pending
4017-1.04	When reporting last known well, state the actual clock time. Do...	4017.1.d (p.151)	ALL	-	Traced; review pending
4017-1.05	Signs & symptoms: facial drooping; arm drift or weakness...	4017.2 Signs & Symptoms (p.151)	EMR+	-	Traced; review pending
4017-1.06	Listed differential: seizure; subdural hematoma; brain tumor...	4017.2 Differential / Pediatric (p.151)	ALL	-	Traced; review pending
4017-1.07	Agencies may, with Medical Director approval, use alternative...	4017.1.e (p.151)	ALL	-	Traced; review pending
4017-2.01	Perform a Cincinnati Pre-hospital Stroke Scale {or alternative...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	-	Traced; review pending
4017-2.02	CPSS (normal or abnormal): facial droop (patient shows teeth or...	4017.2 Clinical Pearls (p.152)	EMR+	-	Traced; review pending
4017-2.03	Perform a Large Vessel Occlusion (LVO) screening looking for...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	-	Traced; review pending
4017-2.04	LVO indicators (Pearls): abnormal findings in all three CPSS...	4017.2 Clinical Pearls (p.152)	ALL	-	Traced; review pending
4017-2.05	Consider the following contraindications to thrombolytics...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	-	Traced; review pending
4017-2.06	Anticoagulant examples listed: Warfarin (Coumadin, Jantoven)...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	Source spellings; see U-4017-7.	Traced; review pending
4017-2.07	Perform blood glucose check (1005 General Patient Management).	1005.4 Tx Algorithm, EMR block (p.22)	EMR+	4017 itself only says 'If glucose is less than 60...' (see U-4017-6).	Traced; review pending
4017-2.08	Time-of-onset / last-known-well interview questions, medication...	-		-	NOT SPECIFIED — VERIFY
4017-3.01	A patient in respiratory distress with pale, moist skin and...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	Interaction with the SpO2 rule is not stated (see U-4017-2).	Traced; review pending
4017-3.02	Be prepared to assist ventilations with OPA/NPA and...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	-	Traced; review pending
4017-3.03	If signs of cerebral herniation are present, ventilate ADULT at...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	Signs of herniation are not defined (see U-4017-3).	Traced; review pending
4017-3.04	PEDIATRIC with signs of cerebral herniation: ventilate at a...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	Wording is ambiguous (see U-4017-3).	Traced; review pending
4017-3.05	{If numeric EtCO2 readings are available, ventilate at a rate...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	-	Traced; review pending
4017-3.06	Never ventilate at less than 8 per minute.	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	-	Traced; review pending
4017-3.07	Patient with indications of stroke and SpO2 less than 94%...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	Exactly 94% is not addressed (see U-4017-1).	Traced; review pending
4017-3.08	Patient with indications of stroke and SpO2 greater than 94%...	4017.2 Tx Algorithm, EMR block (p.151)	EMR+	Exactly 94% is not addressed (see U-4017-1).	Traced; review pending
4017-3.09	If glucose is less than 60, or there is strong suspicion of...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	4008 not reproduced (G-3).	Traced; review pending
4017-3.10	Transport the patient with the bed flat, if able to tolerate...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	-	Traced; review pending
4017-3.11	No additional orders at this level (source prints this for AEMT...	4017.2 Tx Algorithm, AEMT and Paramedic blocks (p.152)	AEMT +	-	Traced; review pending
4017-4.01	Reassessment interval, response criteria and worsening pathway...	-		-	NOT SPECIFIED — VERIFY
4017-5.01	The presence of a single abnormal finding in the CPSS {or...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	See U-4017-5.	Traced; review pending
4017-5.02	Patients with onset greater than 24 hours, clinical findings...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	-	Traced; review pending
4017-5.03	Patients with signs or symptoms that strongly indicate a...	4017.2 Tx Algorithm, EMT block (p.152)	EMT+	Overlap with 'Comprehensive' centers (see U-4017-4).	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
4017-5.04	Contact MCP for Stroke Alerts or for advice regarding transport...	4017.2 Consult (p.152)	ALL	-	Traced; review pending
4017-5.05	Arrange for a historian to be transported with the patient...	4017.2 Clinical Pearls (p.152)	ALL	Source wording is garbled (see U-4017-8).	Traced; review pending
4017-5.06	Stroke Centers (4017.3): Telemedicine Stroke Center (drip and...	4017.3.a-c (p.152)	ALL	-	Traced; review pending
4017-5.07	Receiving-facility notification method / number for a Stroke...	-		-	NOT SPECIFIED — VERIFY
4017-6.01	Record the last known well as an actual clock time (not an...	4017.1.d (p.151)	ALL	-	Traced; review pending
4017-6.02	Other required documentation elements (CPSS results, LVO screen...	-		-	NOT SPECIFIED — VERIFY